

SECTION C: PERFORMANCE WORK STATEMENT

TITLE:

Federal Occupational Health (FOH) Private Provider Network (PPN)/ Contractor-Owned Contractor-Operated (COCO) Client Portal System (CPS).

C.1 Purpose

The purpose of this contract is to acquire a network of occupational health professionals/providers to provide and deliver services through a nationwide private provider network (PPN) to support Program Support Center (PSC) Federal Occupational Health (FOH) under the Department of Health and Human Services (HHS). The nationwide PPN will consist of two regions: FOHNet East and FOHNet West (see Section C.7, Contract Regions for details). The Contractor shall maintain a PPN within the designated FOHNet Region and deliver a full-range of occupational health services, including but not limited to those listed in Attachment F – Service List.

C.2 Background

FOH improves the health, safety, and productivity of the federal workforce. FOH -Clinical Health Services (CHS) delivers a full range of occupational health services to other federal agencies and their employees throughout the United States (U.S.), the District of Columbia, and the U.S. Territories through a large network of private-provider physicians and nurses. FOH provides defined health services as well as consultation to federal customer agencies exclusively through Interagency Agreements (IAAs). FOH is the largest provider of occupational health services in the federal government, serving more than 300 federal entities and reaching more than one million federal employees. Agreements are negotiated with federal departments and federal agencies' national headquarters, or with their regional/local components for products ranging from a standard package of occupational health and safety program components to an array of services, including ambulatory clinical services and expert consultation tailored to a particular federal agency's occupational health and safety, wellness and health promotion, and Employee Assistance Program needs.

FOH sets the standard for promoting a culture of health and safety throughout the federal workforce. The effectiveness of FOH programs depends on the expertise and experience of a Contractor-managed PPN, which provides and delivers direct care through both fixed and mobile service providers. The PPN must:

- Provide high-quality services in individual and group settings.
- Place client/patient needs at the forefront of service delivery.
- Ensure all personnel deliver high-quality individualized care that meets FOH customers' requirements.

Scope

The contractor shall provide all personnel, equipment, supplies, facilities, buildings, transportation, tools, materials, supervision, and other items, and non-personal services, necessary to perform occupational health services, utilizing the contractors private provide network and company owned company operated client portal system, as defined in this Performance Work Statement (PWS) except for those items specified as government furnished property and services. The contractor shall perform to the standards in this contract.

The contractor will complete services for FOH in accordance with work orders and referral documents.

FOH will send appointment referrals to the contractor to schedule clinical services with health care providers. Once examinations are complete, the contractor will return the results of the examination to FOH. When consultation is requested, the Contractor will return the results of the consultation which may include medical evaluation results, responses to medical questions, and the consultant's medical recommendations, as requested, to FOH.

The work performed under this contract requires the Contractor to:

- Accept service referrals (via work orders and referral documents), assign provider(s) who possess the appropriate credentials and expertise to provide the requested service, schedule patients and confirm appointments.
- Ensure all supplies and materials to be used at each scheduled exam including supplies and materials from FOH or other FOH vendors are secured in advance of scheduled exam,
- Complete examinations, retrieve, and compile results for all exam items requested using the forms provided by FOH.
- Provide timely follow up with examinee for all medical test results, lab results, and clinical recommendations per standard of care.
- Perform quality assurance reviews as outlined by FOH of the services provided, including monitoring results for accuracy and completeness.
- Ensure timely submission of the required medical examination and results to FOH in a comprehensive report.
- The ability to securely ship and/or transmit all completed medical record reports to a location(s) designated by FOH.
- The ability to securely store, all FOH medical records per FOH's requirements (e.g., electronic and paper records).
- Notify clients of cancellations or change in schedule within 24 to 48 hours of the original scheduled appointment time.
- The ability to complete all required exams within a specific timeframe and location.

The Contractor will provide and maintain a secure web-based portal or Electronic Health Record (EHR) where FOH can submit patient service referrals and view the real time workflow and status of all referrals, appointments, patient cases, and medical records. The contractor shall submit to FOH for review and evaluation their detailed quality assurance process that will be used for review of medical examinations and records. This review will be part of the evaluation criteria. Examinations, results, forms, and all other patient information, all of which is the property of FOH, shall be uploaded to the Contractor's secure web-based portal. Original documentation may be required to be mailed, shipped, faxed, or otherwise electronically transmitted through secure means to designated FOH location(s).

The Contractor's primary role is to facilitate the delivery of FOH services in the U.S. and its territories where an OHC is unable to perform the required services for a customer. The Contractor will maintain a network of provider locations and staff capable of performing these services within the distance requirements and timeframes specific herein. Where provider locations and staff are not available or able to be established by the contractor, mobile units, are to be made available by the contractor with no additional cost to the government. FOH must approve the use of a mobile unit prior to service delivery.

Provider Service Locations

Service appointments shall be scheduled in the most efficient way so that the examinee shall make the least number of trips to complete the requested examination and testing at a single provider location to the

greatest extent possible. Every effort shall be made to ensure all work order requirements are completed in one visit, unless otherwise stated in the work order. Advance approval from the COR must be obtained if services will be performed at more than one location, or if a second visit is needed due to a failure to complete all requested services on the original work order. Failure to notify and obtain authorization from FOH will result in penalty. In areas where more than one provider location is available within the Contractor's PPN, FOH may dictate which provider location is used to schedule specific FOH customer agencies. FOH may also require that the Contractor discontinue use of a provider at any time if FOH deems the quality of the facility, services, or providers insufficient to meet the needs of the contract. Or, if the provider is unable to follow tenets as set forth in the contract. The COR may require that the use of provider service locations deemed by FOH to be insufficient be discontinued at any time.

If the Contractor is unable to locate a provider, arrangements must be made by the Contractor for requested services to be provided via mobile van when a fixed provider location is unavailable. In such instances, FOH will not be responsible for any charges associated with the mobile van service, hourly outreach practitioner charges, or any charges other than the services provided. The Contractor shall notify FOH in such instances and then obtain approval from the FOH COR to proceed with setting up the mobile service provider(s). The mobile service provider is expected to adhere to the same quality and standards of service as outlined in the contract prior to entering into any provider agreement pertaining to this contract for such service.

New Provider Locations

The Contractor will be expected to verify whether the work order requirements can be satisfied by the Contractor's existing PPN or if a new provider location will be required. When requesting new provider locations, FOH will notify the Contractor of the expected locations that comply with the distance (C.8.1) and work/service order requirements, and estimated volume and timing of referrals, if available. The Contractor shall add new provider locations within the assigned region at no additional cost to the government. If requested location is not covered by the Contractor's existing PPN, the Contractor shall establish a new provider location, or arrange mobile van services, within 30 calendar days of notification from the COR receiving the referral requirements. Failure to establish a location within the required 30 calendar days may result in a disincentive of \$2500.00 each calendar day beyond the 30 calendar days until a new provider location is available for service scheduling. This disincentive shall not exceed \$55,000.00 per new provider location referral. In cases where providers are limited and if the services required cannot be provided via mobile van, the contract will provide a plan to FOH within 3 business days that indicates how the requested service can be rendered.

New Provider Startup Call and/or Visit

The Contractor shall support a new provider location "startup" call" or visit with FOH, FOH agency customers, and any other key stakeholder determined by the COR, when requested. The purpose of the call is to identify, learn, and understand the new provider and unique forms, training, provider- specific logistics, or criteria that will need to be addressed prior to scheduling patient care services with the new provider location. The Contractor shall not be disincentivized for timeline delays for which FOH or FOH customer agencies are responsible (e.g., FOH site visit availability, FOH startup call availability, etc.).

The Contractor shall designate a team of program directors and managers with the necessary qualifications and licensure to provide oversight and be responsible for all Contractor operations. The Contractor will be responsible for receipt of all referrals for service requirements sent by FOH and the coordination and administration of the necessary services. The subcontractors and Contractor employees are expected to

follow the same requirements as the contractor.

The Contractor shall review all examinations, work order results, and associated documentation for quality assurance prior to submission to FOH. FOH may make the final determination as to the quality of the service provided. FOH must have a point of contact who works with the Quality Assurance team to resolve issues regarding the services provided to FOH by the Contractor. Contractor shall designate an individual to receive FOH inquiries and provide responses to issues within 24 hours or one business day.

PPN Services

Service/work order requirements may be performed through a variety of contractor PPN facilities that are capable of administering FOH examinations and services such as, but not limited to, hospitals, occupational health clinics, general medical clinics, specialty clinics, and urgent care facilities. All required orders and forms will be specified in the work orders and made available to the Contractor by FOH in advance of service referrals. The Contractor shall distribute within 1 business day of receipt all updated work order required forms, documents, and supplies to the PPN provider responsible for performing services and provide any needed updates. The Contractor shall ensure annually that they receive from FOH an updated set of examination forms, documents, and work orders prior to the start of each FOH fiscal year.

The PPN provider is responsible for ensuring all clients are appropriately notified of exam and testing results and that the examining physician or licensed independent practitioner provides appropriate clinical and follow-up recommendations, as indicated. This shall be documented in the client's record in the provider's office where the examination was performed as well as in their examination report that is provided to FOH. The Contractor shall ensure the provider is adhering to all FOH notification requirements for FOH and federal agencies related to examinees, examinations, testing results, and mandated reporting. Certain agencies also require notification for abnormal results or post-exposure testing. The Contractor shall ensure all notification instructions included in the work order are followed. The following are examples of notifications that are required:

- The Contractor shall notify FOH of positive Tuberculosis (TB) test results and all post-exposure TB test results (positive and negative) within 24 hours or one business day.
- The Contractor shall ensure providers are following all mandatory infectious disease reporting as required by the federal government, FOH policy, the state, and the Centers for Disease Control and Prevention.

A full list of required notifications will be provided upon award.

Outreach Services

Outreach services are a key source of health care that are used to provide healthcare services in rural communities. Outreach services are also useful to provide services when a large number of employees located in the same area require similar services (e.g., flu vaccinations). Outreach services shall take place at qualified facilities and with qualified personnel. Outreach services include services that are provided by mobile vans and traveling medical staff who are equipped with appropriate medical equipment and supplies.

Qualified facilities are locations that comply with all federal, state, and local healthcare regulations,

guidelines, standards and other recognized best practices, maintain proper licensing and certifications and comply with The Joint Commission standards. The outreach health care services must be delivered by appropriately certified, credentialed, and licensed personnel.

FOH will indicate on a referral when outreach services are required or when a customer referrals services at a specific location. FOH will also notify the Contractor about work order requirements for outreach services. The outreach facilities must ensure patient privacy in accordance with HIPAA and the Federal Privacy Act, as well as provide safe and suitable environments for providing occupational health services that meet medical standards of care. This requirement also applies to mobile units and temporary setups staffed by trained professionals when fixed facilities are not feasible.

The Contractor must receive approval from the FOH COR prior to scheduling outreach services. The FOH COR will send the Contractor a referral for outreach services (e.g., audiograms, vaccines, flu shots, etc.) electronically along with the applicable work order. The Contractor is required to provide a cost estimate to FOH, including possible dates of service, prior to scheduling. Thus, the Contractor's quote must specify an expiration date.

Mobile Van Outreach Fee

The mobile van outreach fee is utilized when the work order requirements indicate a mobile van outreach service is required. The fee will be billed upon completion of all work order requirements contained within the referral. The fee is intended to cover non-service-related costs incurred by the Contractor – these costs may include but are not limited to a mobile van, setup/cleanup, travel expenses, and/or driver (if other than service practitioner). The fee is intended to be applied on a “per hour/event” basis. Individual services performed during a mobile outreach event shall be invoiced in addition to the Mobile Outreach Fee. Please see C.15.5 Mobile Outreach Fee for additional information.

Mobile Van Outreach Late Cancellation Fee

The Contractor can charge FOH a mobile van outreach late cancellation fee for failure to appear or cancel within three (3) business days of the scheduled event date. The fee is intended to cover non-service-related costs and applied on a “per day/event” basis.

Quality Assurance

The Contractor shall implement a quality assurance process to review all materials for accuracy and completeness. The quality assurance process shall include:

- Monitoring all exam components and forms, including required signatures and dates, to demonstrate that testing is adequate, and that all information is captured accurately and reviewed in accordance with the instructions provided in the work orders.
- A process and accompanying documentation demonstrating that all providers and their staff are properly trained, certified, and licensed to deliver the required services. For example, providers must be trained on FOH policies and procedures, appropriate forms and documents to complete, and hold professional certifications.
- Verification that all reports, results and documents from requested services undergo quality

review. The contractor shall notify FOH of any missing examination items or delays within 24 business hours of the exam appointment. This notification must include the reason for the missing items or delays. In the rare occasion, if the examination has a missing item(s), the contractor may referral additional time but must obtain approval from FOH before proceeding.

- Tracking and maintaining documentation of the transmission of service results to FOH. All documentation and service recommendations shall be submitted to FOH within five business days following the completion of the examination.

Health Care Staff Credentials

All practitioners and professionals engaged under this contract must meet the following requirements:

- Education
 - Hold degrees from accredited institutions in their respective field (e.g., MD, DO, NP, RN, Audiologist, etc.).
 - Complete specialized training required to perform occupational health services, such as required occupational health examinations, audiometry, spirometry, travel medicine, or vaccinations.
- Expertise
 - Demonstrate proficiency in delivering occupational health services, including preventive care, health assessments, and wellness programs.
 - Maintain current knowledge of federal health regulations, The Joint Commission (TJC) standards, and best practices in occupational health.
- Work Experience
 - Possess relevant clinical experience in occupational health or related healthcare settings.
 - Have documented experience providing services in various environments, including fixed facilities and mobile outreach settings.
- Licensure and Certification
 - Hold and maintain active state licensure for their profession throughout the contract period.
 - Maintain certifications such as Basic Life Support, other credentials as required by service type.
- Continuing Education
 - Participate in ongoing professional development and training to ensure compliance with evolving standards and regulations.
- Compliance and Ethics
 - Adhere to HIPAA and federal privacy regulations.
 - Follow FOH policies and ethical standards for patient care and safety.

FOH is committed to delivering safe, high-quality care to our customers and partner agencies by adhering to The Joint Commission Standards for Ambulatory Care, including the requirements for credentialing and privileging of licensed independent practitioners and other clinical staff. FOH has invested

significant time and resources in aligning its policies, procedures, and operational practices with TJC guidance and standards for credentialing, privileging, and assessing staff competency.

The Contractor shall ensure that all testing and services provided under this contract comply with TJC Standards for Ambulatory Care, including all applicable human resource requirements. The Contractor shall support FOH in its processes to maintain compliance with TJC ambulatory care standards and by actively participating and contributing to FOH's performance improvement initiatives.

FOH Policy

The quality of services provided by the Contractor shall be guided at a minimum by national healthcare quality standards recognized by relevant national medical specialty boards, established industry best practices and ethics, and the standards of care in the state or locality where the services are provided.

FOH maintains a robust quality, safety, and risk management (QSRM) program to provide and maintain a safe, functional, supportive, and effective environment for customers. The Contractor shall proactively promote a culture of safety by ensuring that all services comply with the national standards referenced above, maintain processes for safety event data collection and reporting, participate in performance improvement activities, administer staff surveys, and support patient safety through effective communication practices with FOH.

C.3 Challenges to be mitigated

The Contractor can expect to face several major challenges throughout the life of this contract. These challenges if not properly managed by the Contractor may result in the Contractor not providing the services required in this PWS and may result in serious negative repercussions to the Contractor as well as jeopardy of FOH's mission. Key challenges associated with this contract that require the Contractor's attention include:

- **Efficient Communication:** The Contractor is expected to communicate and coordinate efficiently with its PPN providers to ensure timely completion of exams and tests as directed by FOH.
- **Missing and Delayed Items:** The Contractor is expected to have contingency plans for missing or delayed items to minimize collection of repeat samples for testing.
- **Scheduling process:** The Contractor must ensure communications facilitate timely scheduling, incorporate an appointment confirmation- process, and address facility closures to help prevent missed or rescheduled visits. This involves identifying an accurate and complete list of required testing for a client prior to scheduling, ensuring necessary equipment and skilled personnel will be at the appointed facility to complete the exam, and submitting complete reports to FOH within the specified timeframe(s).
- **Records management:** The contractor is expected to oversee the secure and confidential management of federal medical records (paper and electronic) in accordance with Office of Personnel Management (OPM), the National Archives and Records Administration (NARA), and the Office of the National Coordinator (ONC) requirements for electronic and physical medical records.

- **Efficient Billing:** The Contractor must ensure proper and timely billing for FOH services through effective and coordinated communication with its PPN providers, thereby preventing improper or misdirected billing. This includes establishing a clear process for identifying FOH clients to ensure charges are directed to the appropriate vendor, rather than accepting clients' insurance cards or issuing bills directly to clients.
- **Service Provision in Medically Underserved Areas:** The Contractor is expected to ensure that FOH clients receive evidence based care that complies with federal regulations, national healthcare quality standards, and industry best practices, consistent with FOH's location requirements. This includes delivering services within required timeframes in rural, sparsely populated areas, and provider desert locations.
- **Reporting Medical Information:** The contractor is expected to report abnormal labs and testing results to FOH and clients when necessary and within required timeframes in compliance with applicable federal privacy and record disclosure regulations. The provider must offer follow-up recommendations as needed and document these actions appropriately.

The Contractor must ensure that the PPN providers in their network understand the process for communicating examination and medical test results to the contractor. The Contractor should also establish a process to ensure PPN providers send examinations and test results to the correct recipients.

The contractor shall submit a mitigation plan for all above challenges within 30 days of the effective date of the contract award.

C.4 Definitions/Acronyms/Abbreviations

- ATO – A government acronym for Authority To Operate.
- Contracting Officer's Representative (COR) - An individual within FOH appointed by the Contracting Officer to manage and monitor this contract from a technical standpoint.
- CUI – Controlled Unclassified Information
- CONUS – A government acronym for the contiguous US.
- OCONUS - A government acronym referring to locations outside the contiguous U.S., including areas beyond the 48 adjoining states and the District of Columbia. This encompasses foreign countries as well as U.S. states (Alaska, Hawaii) and territories such as Guam and Puerto Rico.
- Contractor Secure Web-Based Portal - A Contractor owned and contractor operated (COCO) secure web-based portal with an HHS issued ATO to be used as an interface to manage FOH client data. The portal allows selected FOH and agency personnel to access real-time client reports and data. The portal also allows select FOH federal employees to review reports and invoicing data. The portal must have the minimum capabilities listed below:
 - Examinee self-scheduling capabilities
 - Date and Type of Referral
 - List of Tests Required on individual client

- Date of Examination
 - Date of Exam Cancellation
 - Date of Exam Reschedule
 - Date Exam went for Quality Review
 - Whether Exam has Missing Item and Examinee Notification
 - Individual Exam Reports including Outreach reports
 - Case Notes regarding delays and pertinent information
 - Reports by Agency
 - Reports by Date
- Federal Occupational Health (FOH) - A service component within the Department of Health and Human Services offering comprehensive occupational health programs to improve the health, safety and productivity of the federal workforce. FOH's governing organization is the Program Support Center (PSC), within the Office of the Assistant Secretary of Administration.
 - FOH Customer - A federal agency who has entered into an inter-agency agreement with FOH to have their federal employees receive services from FOH at a Service Provision Site (SPS).
 - FOH Private Provider Network (FOHNet) - A network of private provider facilities used by FOH to expand access to services beyond its SPSs or OHCs to meet the health and wellness needs of Federal government employees.
 - IME – Independent Medical Examinations
 - NIST – A government acronym for National Institute of Standards and Technology.
 - NPI – An acronym for National Provider Identifier.
 - Mobile Outreach Services – On-site or off-site clinical and occupational health services delivered by FOH or its Contractor by their healthcare professionals at designated federal agency locations or in the field.
 - Mobile Van – A Contractor-operated vehicle equipped to deliver clinical and occupational health services directly at or near federal worksites and/or community locations.
 - Occupational Health Centers (OHCs) - FOH managed health clinics and wellness centers located throughout the U.S., offering occupational health and wellness services to Federal government employees. FOH has clinics and wellness centers located outside of the continental U.S. (e.g., Guam, Hawaii, Alaska, Puerto Rico, etc.)
 - Outreach Services - Clinical services that are provided at the employee's worksite or other agency-requested location at a FOH-approved site FOH. Typical services offered as outreach services include immunizations and screenings. Outreach services may be provided using mobile van services.
 - PII – A government acronym for Personally Identifiable Information.
 - PPN – Private Provider Network

- Service Provision Sites (SPSs) - FOH sites capable of providing occupational health and wellness services. These include FOH Area Offices, FOH OHCs, FOH Fitness Centers and FOH Private Provider Network (FOH PPN) sites.

Work order - A document with specific information and instruction for health and wellness services that an agency authorizes for their federal employees. A Work Order is part of an FOH referral for services. The work order includes a checklist of approved services based on agency requirements, as determined by the Interagency Agreement (IAA), that may include scheduling instructions. The work order is not specific to a particular Federal government employee.

C.5 Place of Performance

Contract services shall be carried out within the continental U.S. (CONUS) and outside the continental U.S. (OCONUS), as specified in the contract.

C.6 Contract Regions

The Contractor will be responsible for all PPN referrals within the following four regions:

- NORTHEAST: Connecticut(CT), Maine(ME), Massachusetts(MA), New Hampshire(NH), New Jersey(NJ), New York(NY), Pennsylvania(PA), Rhode Island(RI), Vermont(VT)
- MIDWEST: Illinois(IL), Indiana(IN), Michigan(MI), Ohio(OH), Wisconsin(WI), Iowa(IA), Kansas(KS), Minnesota(MN), Missouri(MO), Nebraska(NE), North Dakota(ND), South Dakota(SD)
- SOUTH: Alabama(AL), Arkansas(AR), Delaware(DE), District of Columbia(DC), Florida(FL), Georgia(GA), Kentucky(KY), Louisiana(LA), Maryland(MD), Mississippi(MS), North Carolina(NC), Oklahoma(OK), South Carolina(SC), Tennessee(TN), Texas(TX), Virginia(VA), West Virginia(WV), Puerto Rico, U.S. Virgin Islands
- WEST: Alaska(AK), Arizona(AZ), California(CA), Colorado(CO), Hawaii(HI), Idaho(ID), Montana(MT), Nevada(NV), New Mexico(NM), Oregon(OR), Utah(UT), Washington(WA), Wyoming(WY), American Samoa, Guam, Northern Mariana Islands

Attachment D – Service Region Map contains an illustrated map of the Regions. The Contractor will be expected to maintain a PPN network capable of servicing all regions at a moment's notice. The government shall have the unilateral right to adjust the regions on a Task Order based on performance. The Contractor shall maintain a single price for each service on this contract that will be valid nationwide. The "OCONUS Fee" identified in *C.15.3* applies when a new service location is added outside the continental United States.

Distance Requirements

In general, referrals for work/service order requirements in cities/towns with populations under 100,000 should be within a driving distance of 50 miles of the patient/employee's location. The Contractor shall notify FOH when it is unable to meet the 50-mile requirement. The Contractor shall submit details to FOH in writing the actions taken by the Contractor to locate a provider location within the specified distance

requirement, and detailed information on the closest available provider location. The COR may assess a disincentive up to 20% of the total cost of all work order service requirements per patient/employee referral for any service(s) performed outside the distance requirements without prior COR approval. The distance requirements may be waived on a case-by-case basis, with approval from FOH.

Work order requirements may specify different distance requirements for provider location(s) than identified in the paragraph above. The Contractor shall make every reasonable accommodation and shall not be subject to prior approval from the COR or a disincentive prior to scheduling the requested service when a service scheduled using distance requirements as specified in the applicable work order.

C.7 Overview of Services

Work order requirements must be completed by licensed health care provider(s)(LHCP) whose knowledge, skills, and abilities are commensurate with the scope of practice per the state or territory requirements in which the service is performed. All LHCPs must be board-certified physicians with either a MD or DO degree unless otherwise specified by FOH.

All tests and services need to be performed using accurate, properly calibrated and maintained equipment, and properly trained and credentialed personnel familiar with the required occupational health standards for testing and services. The tests and exams must be performed exactly as outlined per FOH and reviewed for the quality and accuracy of examination and testing, and will include accurate, legible, and complete documentation.

Audiometry equipment and calibration shall meet all requirements set forth in OSHA Standard 1910.95 Occupational noise exposure, and audiograms must be performed and reported as specified in FOH Work Orders. Performance of spirometry shall meet National Institute for Occupational Safety and Health and the American Thoracic Society's latest standards. FOH reserves the right to referral calibration and maintenance documentation from the Contractor for any service provider location or mobile provider. Calibration documentation shall be provided to the FOH within five (5) calendar days of the referral.

The Contractor is responsible for ensuring that necessary training and instructions are given to all PPN personnel dealing directly with FOH examinees, including but not limited to updates in FOH policies, procedures, documents and forms and information regarding penalties for late exams, missing items, and improper billing. Work order requirements may identify services individually or be included as part of customized physical exam referral(s). FOH will initiate all referrals for the Contractor's PPN to provide service.

The Contractor shall maintain and temporarily store all records and results. The Contractor must submit all results to FOH electronically through the Contractor's secure web-based portal. All electronic and paper medical records associated with the Contractor's PPN shall remain the property of FOH. The Contractor and its providers are prohibited from permanently storing paper originals or copies at any location.

The Contractor shall not use electronic medical records outside the scope this contract or for any purpose unrelated to the provision of clinical services under this agreement.

Attachment F – Service List Supplemental Information - The CPT/CDT codes and volume estimates included in *Attachment F* are designed to assist with initial line item pricing only. The service titles and descriptions provide a general summary of each line item. Additional requirements and instructions for each line item may be included in the individual work or service order, as specified by FOH or the

applicable FOH customer agency policy.

Specialized Services

Work order requirements will be clarified in each referral provided to the Contractor. Work order requirements may also include additional referrals for specialized services. Examples of specialized services include tailored operational process flows for scheduling and service timing, specialized services or examinations, use of agency specific forms, and specific practitioner credentials and certifications. Further clarification beyond the work order requirements may be obtained from FOH upon referral.

Management of the Private Provider Network

The Contractor shall maintain a PPN network and provide FOH with a comprehensive list of all active provider locations with associated practitioners and service capabilities. The complete list will be provided to the COR within 30 days from the effective date of the task order or upon referral from the COR. Thereafter, updates to PPN locations will be provided to the COR within 3 business days of FOH requesting a PPN change.

Laboratory Services

The Contractor shall prepare laboratory requisitions for FOH examinees and ensure that each client is directed to FOH's designated laboratory vendor prior to their scheduled appointment. The Contractor shall ensure that each examinee is provided a minimum of seven (7) calendar days to complete the required laboratory work prior to their scheduled appointment.

Vaccines

The Contractor will prepare and ship vaccines, when required. Shipment of vaccines shall meet or exceed Centers for Disease Control and Prevention (CDC) requirements for vaccine shipment. The Contractor will be responsible for shipping, faxing, and electronically transmitting medical records to specified locations as identified in the work order or as directed by FOH.

Complaint Resolution Plan

The Contractor's PPN staff shall conduct themselves in a professional and appropriate manner at all times, including in their interactions with patients and in maintaining equipment quality, facility cleanliness, and other related responsibilities. The Contractor shall monitor and manage PPN performance. Providers who fail to meet the requirements of a referral may be subject to removal from providing services to FOH examinees at FOH's request.

FOH may also require the Contractor to develop a Complaint Resolution Plan. The Contractor must review all service and provider complaints related to inappropriate or unprofessional conduct and poor performance. All complaints that the Contractor receives about services provided under this contract must be submitted to the COR along with a plan for resolution within five (5) business days of the Contractor receiving the complaint. Failure to submit or resolve complaints within 30 business days may result in a negative performance review and escalation of the issue to the Contracting Officer for disciplinary action.

Provider Site Visits

FOH reserves the right to conduct site visits at locations that provide services for this contract. The COR will contact the Contractor's Program Manager to schedule site visits. Site visits may include a detailed evaluation of the provider's facility by FOH designated personnel. At any time during the life of the contract, the COR may instruct the Contractor to discontinue using a provider location(s) to perform services for this contract.

C.7.1 Web-Based Portal Capability

The Contractor shall receive all referrals through the Contractor's secure web-based portal. FOH will place a referral into the Contractor's secure web-based portal for action. It is the Contractor's responsibility to check their secure web-based portal and manage each record in accordance with the timeframes outlined within the PWS.

The Contractor is responsible for transmitting patient information to FOH by uploading service results, lab results, forms, and any other required information into the Contractor's secure web-based portal.

The Contractor shall provide a secure web-based portal that provides FOH staff the following capabilities:

- Ability to upload individual and batch appointment referrals into the secure web-based portal.
- Allow FOH to edit a referral after submission. The portal must track and report information about the edits including the date, time, and identity of the person making the changes.
- Search for examinees and view the status of referrals for services, appointments, and cases within the system.
- Create, retrieve, and download management reports containing case information. For example, the portal must be able to generate reports showing referral statuses such as pending scheduling, scheduled/open, completed, cancelled, and rescheduled, along with the corresponding dates. Reports must include specific details such as processing milestone dates and times and automatically calculate duration.
- Capability to export the data in Excel format. The type, frequency and format of the data will be determined by FOH.
- Retrieve completed exam and laboratory results.
- Create reports with detailed charges associated with a monthly invoice.
- Create reports with FOH-specified quality indicators monthly.

If the referral does not contain sufficient information required to complete the referral, then the Contractor must notify FOH within 24 hours of receipt of the referral. FOH will provide the requested information and edit the referral within the Contractor's secure web-based portal. The Contractor shall complete the assigned work while adhering to the requirements contained this PWS.

Records Management and Retention

The Contractor and any subcontractors must maintain all employee medical information in compliance with Executive Order 13556 (Controlled Unclassified Information), the federal Privacy Act, OPM guidance, OPM/GOVT-10 Employee Medical File System Records requirements, NARA

General Records Schedules (GRS), and HHS and FOH policies. They must not dispose of, transfer, or alter any medical records without prior written authorization from HHS/FOH. All records must be retained and disposed of according to applicable GRS and agency-specific schedules to ensure confidentiality, integrity, and retrievability throughout their lifecycle.

At the end of contract performance, the COR will provide disposition instructions to the contractor for securely transferring or destroying in accordance with 36 CFR § 1226.24 and NIST SP 800-88. These regulations apply to any records that are in the contractor's (or any subcontractor's) custody or control and subject to the federal Privacy Act that were created or maintained to perform services for this contract .

Under no circumstances may the Contractor or any PPN location destroy, delete, compromise, or disclose medical records except as directed by the COR and in accordance with the Performance Work Statement (PWS).

The Contractor and any subcontractors must immediately document and report any accidental disposal or destruction of medical records to FOH in accordance with HHS/FOH policy.

Loss or Compromise of Records

The Contractor and any shipping company utilized by the Contractor must notify FOH within 24 business hours of discovering any loss, misplacement, or compromise of records, forms, or other documents containing examinee information. The notification must include, at a minimum:

- Names of all affected examinees
- A description of the incident
- The type of information involved
- Circumstances surrounding the loss

Credit Monitoring

The Contractor must provide credit monitoring services for three (3) years to all affected examinees when PII or protected health information is compromised or disclosed at no cost to the Government or the federal employee.

Corrective Action Plan

Within 72 business hours of discovery, the Contractor must submit a written corrective action plan to FOH. The plan must include:

- Root cause analysis
- Mitigation strategies
- Measures and an implementation timeline to prevent recurrence

Confidentiality of Medical Records

The Contractor shall treat all records containing employee medical information as confidential and maintain them in accordance with:

- 5 U.S.C. 552a, the Privacy Act of 1974

- Federal regulations
- OPM/GOVT-10 Employee Medical File System Records requirements
- FOH policy and procedures
- Executive Order 13556 (Controlled Unclassified Information)
- NARA GRS

Health Insurance Portability Accountability Act (HIPAA) Compliance

HHSAR Subpart 324.70 – HIPAA of 1996 applies to this requirement. The Contractor shall comply with HIPAA in accordance with this subpart and 45 CFR 160 and 164.

Privacy Act Applicability

This contract is subject to the Privacy Act of 1974 because it involves the design, development, or operation of a system of records on individuals. Performing federal occupational health services requires Contractor PPN staff to have access to PII and PHI under federal agency control. The applicable System of Records Notices (SORNs) include:

- OPM/GOVT-10 Employee Medical File System Records, June 21, 2010, 75 FR 35099, updated November 30, 2015, 80 FR 74815 (covers medical records about civilian federal employees)
- OPM/GOVT-5 OPM/GOVT-5, Recruiting, Examining and Placement Records
- Similar SORNs published by any FOH customer agencies that are independent agencies, defense agencies, etc., whose employees are hired under a non-OPM hiring authority.

Emergency and Surge Services

FOH may require the Contractor to provide emergency and surge services to complete referrals for services that are beyond an estimated volume. The Contractor shall maintain emergency and surge capabilities in place to accommodate such referrals should FOH have the need. When emergency or surge services are invoked, the Contractor shall meet service requirements in accordance with the tenets of this contract, prevailing evidence-based medical standards of care and health care industry best practices.

Under emergency or surge circumstances, the COR will notify the Contractor of the specific service(s) required. The Contractor will notify FOH within 24 hours of its plan to meet the requirement. Changes related to the contingency plan should be communicated to FOH within 24 business hours.

C.8 Services

A. Scheduling

The contractor must receive referrals containing work order requirements through the Contractor's secure web-based portal. The Contractor shall work directly with FOH, FOH customer POCs, or customer employees/applicants, as designated by FOH, to schedule appointments,. The Contractor shall not schedule or complete any services without first receiving a referral from FOH authorizing the services. The Contractor shall make direct contact with employees/applicants, or FOH customer scheduling POCs, within 48 hours of the referral receipt date annotated within the Contractor's secure web-based portal. The Contractor shall make at least three (3) attempts, with at least four (4) hours between each attempt, via phone to contact an applicant, employee, or FOH-specified customer POCs to schedule the appointment.

The Contractor shall continue to attempt to contact the applicant or employee every 72 hours and document all communication attempts and results in the applicant or employee record in the Contractor's secure web-based portal. The Contractor shall maintain a single toll-free phone number to be used for all return calls from FOH and FOH customers. All outbound and inbound calls shall be answered using an identifier established by the COR. If the Contractor is unable to contact the employee after four attempts, then the Contractor shall notify FOH for further instructions. The referral is to remain open until FOH provides instructions for next steps.

The contractor shall submit on a monthly Self-Assessment Report a list of all work order requirements that could not be scheduled due to applicant, employee, or Contractor PPN provider availability. If attempts to communicate with the requester are successful, the Contractor shall provide the applicant or employee with an appointment confirmation notice via email and phone, immediately after scheduling. The email appointment confirmation shall include, at a minimum, the provider's physical address location, practitioner's name, and any special instructions. The content and structure of the appointment confirmation notice email may be changed by the COR at any time. The Contractor shall document the date and time of the phone conversation with the applicant or employee, including whether the individual accepted or declined the appointment. If the Contractor is unable to reach the applicant or employee by phone, then the Contractor shall document their unsuccessful attempt to confirm the appointment by phone including the date, time, whether a message was left on voicemail or with a person, preferably with a name.

Rescheduling Appointments

The Contractor shall ensure that providers have all necessary forms, supplies and materials to complete scheduled appointments in advance. If an examinee arrives for an appointment and must be rescheduled without advance notice, then the Contractor shall bear full responsibility. Any provider who is not prepared or is unable to provide a referred service without notifying the examinee at least 48 hours in advance of the scheduled appointment, will result in the Contractor incurring a penalty of \$1,000.00 per rescheduled appointment.

If an appointment must be rescheduled, the Contractor shall document the reason, specifying whether it was due to the employee, the provider, or another cause. The Contractor must attempt to reschedule the employee within 48 hours of the original appointment. If the Contractor is unable to reschedule the employee, then FOH must be notified in writing within 24 hours. The referral must remain open until FOH provides instruction to close the referral. The Contractor must document rescheduled appointments or additional appointments needed to complete all work order requirements in the Contractor's secure web-based portal and in the monthly report Self- Assessment Report.

B. Standard Referral Service Completion

The Contractor shall schedule and complete all work order requirements within 10 business days of referral receipt. The Contractor must confirm receipt of referrals within 48 business hours of FOH submitting the referral. If the Contractor does not schedule and complete all required services (excluding those for the Medical Employability Program) within 10 business days from confirming receipt of the referral, a \$500.00 disincentive will be applied. Medical Employability Program referrals must be completed within 30 business days. If the Contractor is unable to complete the work order requirements for the referral within 10 or 30 business days, as indicated in this paragraph, then the Contractor must contact FOH immediately for further instructions. The Contractor must explain to FOH in writing why the work is unable to be completed within the required timeframe and submit a written contingency plan.

C. Expedited Referral Service Completion

The Contractor shall schedule and complete all expedited referral work order requirements within five (5) business days of referral receipt. FOH will mark the referral as “Expedited” in the Contractor’s secure web-based portal. Failure to schedule and complete all work order requirements within five (5) business days from referral date will result in forfeiture of the Expedited Service Fee (C.15.4) and the Standard Referral Service Completion (C.10.2) fee shall apply.

D. Standard Referral Report Submission

Once all requirements listed on the work order in each standard referral are completed, the Contractor shall compile the information (e.g., test results, procedures, completed forms, etc.) and match it to the corresponding referral, perform quality assurance, and submit the completed referral to FOH within 10 business days. Failure to submit the completed referral back to FOH within 10 business days from the final appointment completion date or notify FOH regarding issues that prevent the Contractor from meeting this requirement Contractor notifications to FOH regarding referral completion issues should explain the issues preventing referral completion well as steps that the Contractor will take toward resolution. The Contractor will take measures to complete the examination within seven business days from the date of identification of the quality review.

Following the submission of a report, FOH-reviewing physicians may need to contact PPN practitioners directly for clarification regarding the services provided or any issues presented by the Contractor. The Contractor shall support and facilitate this direct communication within all PPN agreements within 24 hours of referral by FOH and at no additional cost. The Contractor shall continue to facilitate communications until all issues and/or questions between FOH and the PPN are resolved and the employee’s case is closed.

E. Expedited Referral Report Submission

Once all requirements listed on the work order within each expedited referral are completed, the Contractor shall compile the information (e.g., test results, procedures, completed forms, etc.) and pair it with the appropriate referral, perform quality assurance and submit the completed referral back to FOH via the Contractor’s secure web-based portal within five (5) business days. Failure to submit the referral back to FOH within five (5) business days from the final appointment completion date will result in forfeiture of the Expedited Service Fee (C.15.4) and the Standard Referral Report Submission (C.10.5) requirements shall apply.

Following the submission of a report, FOH reviewing physicians may have a need to contact PPN practitioners directly for clarification regarding the services provided or issues presented by the practitioner. The Contractor shall support and facilitate this direct communication within all PPN agreements within 24 business hours of referral by FOH and at no additional cost. The Contractor shall continue to facilitate communications until all issues and or questions between FOH and the PPN are resolved and the employee’s case is closed.

F. Missing Items

A missing exam item can consist of any item that is omitted and required to complete a service. Examples of missing items include a missing: service, signature, incomplete form, technically inadequate or uninterpretable results or unanswered questions. A referral that is submitted to FOH as “final” and

complete, but it contains one or more missing items will be considered “incomplete.” FOH will return the Contractor’s report for the service to the Contractor with all missing items identified. Applicable timeframes annotated in this PWS will resume, including any associated disincentives. Missing items identified by FOH must be resolved within five business days. If the Contractor is unable to meet the five (5) business days requirement, then the Contractor shall notify FOH immediately with explanation. Failure to meet the five business day requirement will result in a fine of \$500 per missing item.

C.9 Examination Services

The Contractor shall perform occupational health exam services in accordance with *Attachment A*, as requested. The Contractor shall send all required forms listed in the work order directly to the customer and provider at least 2 business days prior to all appointments. FOH requires that all service results, lab results, forms, and any other required information be digitally submitted within the Contractor’s secure web-based portal. In some instances, FOH customers may require hardcopy originals or copies of originals. The Contractor shall ship original documents that FOH requests to designated FOH locations at the Government’s expense via the Government’s preferred carrier. Prior to shipping the document, the Contractor must submit a complete, accurate digital version of the record to FOH via the Contractor’s secure web- based portal.

When FOH requests examination components that require additional appointments with multiple providers (such as x-rays or cardiac services), these appointments should be scheduled concurrently whenever possible. This is intended to reduce the number of visit days and the time between appointments. However, due to patient availability, formal reports, documentation, and results may not always be complete and ready for submission to FOH within the required timeframes. In such instances, the employee’s record should be properly annotated by the Contractor and FOH notified in order to avoid late and missing item penalties. While awaiting outstanding results, the Contractor should submit preliminary reports with the results of completed items in accordance with the contractual timeframes as outlined. The Contractor must submit written notification and information about pending results with its preliminary report. All exceptions to the stated timeframes must receive prior FOH review and approval. Justification shall include the reasons for the delay and the expected delivery date. Failure to present valid reason, as determined by FOH, for the delay shall result in a penalty to be accrued each business day that a referral result is late beyond the stated timeframe requirements in the contract.

A. Specialist Examinations

The Contractor’s web-based portal must allow FOH to submit secure referrals for board-certified specialists to be submitted as individual referrals. Because board-certified specialists can be difficult to locate, particularly in remote areas, a one-way mileage allowance of up to 100 miles is permitted. The distance waiver requirements outlined in Section C.8 will still apply. These services can include the specialties listed in Attachment F.

The Contractor shall make every attempt to schedule specialist appointments within five business days of receipt of the referral. Failure to schedule and complete all services within five business days of referral will result in a penalty of \$125.00 for each business day beyond the five business days. The Contractor will not be penalized should the customer choose to schedule an appointment beyond five business days.

Following completion of the referral, Standard Referral Report Submission (C.10.5) requirements shall apply. To avoid unintended penalties, these exceptions should be documented within the

Contractor's secure web-based portal and Self-Assessment Report.

B. Independent Medical Examinations (IME)

Independent Medical Examinations (IMEs) are required to be performed by board-certified physicians unless otherwise stated in writing by the COR. The most common reasons are Fitness-for-Duty (FFD), Return to Work, or medical consultation required to complete types of medical reviews such as reasonable accommodation requests. A more detailed assessment by another physician may be requested based on the results of the IME.

The Contractor must forward to the provider all documents FOH submits with the referral for medical review. The contracted physician shall provide a well-summarized report to FOH with clear responses for each service requirement in the referral. The specialist is required to review and address all questions and requests for medical testing or documentation before submitting the IME referral report to FOH. Medical documentation includes, but is not limited to, current medical records, prior treatment records, job descriptions, prior medical evaluations, diagnostic testing, and questions to be answered by the specialist in their final report. After submission of the final report in the Contractor's secure web-based portal, the Contractor must destroy any medical documentation it received that is not necessary to justify the final medical report.

C. Psychological Evaluations

Psychological services shall be conducted by a Psychologist or board-certified Psychiatrist. Additional certifications, licensure, and experience such as neuropsychology may be required depending on FOH IAA service requirements. The psychological evaluation can include completion of psychological tests to determine fitness for duty or job suitability for law enforcement positions using FOH procedures and report templates. The price of these services for the subsequent option period(s), and the applicable cancellation fees (C.15.1), shall be renegotiated prior to exercising an option period.

The following provides detail on the main types of psychological evaluations FOH will refer.

- Psychological Fit-for-Duty – To evaluate an employee's psychological health in relationship to their ability to perform the essential duties of their job safely and efficiently. The service is performed by an appropriate psychologist and includes a clinical interview, mental status exam and behavioral observations, review of relevant records, assessment of functional work limitations and identification of restrictions when applicable, and objective psychological testing including scoring and interpretation. attitudes.
- Psychological Pre-Placement - To assess the psychological suitability of candidates for law enforcement or other disciplines. The service is performed by an appropriate psychologist.
- Psychological Security Clearance - The Psychological Security Clearance service is similar to the Psychological Fit-for-Duty with questions and criteria focused on national security. The service is performed by an appropriate psychologist and includes a clinical interview, mental status exam and behavioral observations, review of relevant records – including the FSF-86 Questionnaire for National Security Positions which has been completed by the employee – and objective psychological testing including scoring and

interpretation.

- Neuropsychological Testing - Neuropsychological tests are specifically designed tasks used to measure a psychological function known to be linked to a particular brain structure or pathway. Neuropsychological testing is typically performed in areas, with more than one test per ability area. These ability areas include skills such as memory, attention, processing speed, reasoning, judgment, and problem- solving, spatial, and language functions. The entire battery typically requires at least 6 hours for administration, but can take up to eight hours, depending on the complexity of the issues to be addressed by the evaluation and the individual's condition.

Examiner Qualification requirements for Pre-Employment and other work-related psychological evaluations

- Pre-employment and other work-related psychological evaluations should be conducted only by a licensed, doctoral-level psychologists or board-certified psychiatrists, as indicated.
- Demonstrated competence based on their education, training, supervised experience, consultation, study, and professional experience. Demonstrated expertise in clinical assessment employees for work-related psychological evaluations as well as in the assessment of normal personality characteristics, skills, and abilities relevant to personnel selection.
- Commitment to developing their areas of expertise.
- Trained and experienced specifically in the provision of pre-employment psychological evaluations or other referred psychological evaluation types for federal employees including federal public safety employees.
- Participates in regular (within the past 18 months), ongoing continuing education and training that is specific to the referred evaluation type for federal employees.
- Bases their opinions on established scientific and professional knowledge of the discipline and is knowledgeable about the research available literature that is relevant to psychological testing for public safety positions.
- Demonstrate proficiency with employment law impacting the conduct of pre-employment psychological evaluations, particularly for federal employees, including but not limited to 5 CFR 339, the ADA, ADAAA, GINA, and other federal and state laws applicable to the practitioner's jurisdiction. Examiners should consult with legal counsel when appropriate.
- Adheres to ethical principles and standards for practice in their profession and jurisdiction.
- Proficiency with conducting pre-employment psychological evaluations, preferably for federal agencies, in accordance with current International Association of Chiefs of Police and American Psychological Association pre-employment evaluation guidelines.
- Familiar with, and be guided by, other applicable professional guidelines, including, but not limited to, the Specialty Guidelines for Forensic Psychology.

- Satisfy requirements imposed by local jurisdiction or law.
- When an FFDE is known to be in the context of litigation, arbitration, or another adjudicative process, it is suggested the examiner be prepared by training and experience to qualify as an expert in any related adjudicative proceeding.

C.9.2 Independent Medical Reviews

Independent Medical Reviews (IMRs) are required by certain federal agencies. These services require a board-certified specialist expert in a relevant field to review medical records and answer questions from FOH that are submitted with the referral. The questions are usually to ascertain the extent of a claimed condition, to determine cause or impairment, or to evaluate a medical condition in the context of a reasonable accommodation request. These referrals usually do not require a face-to-face visit with the patient. Sometimes, FOH will request additional medical testing for the independent review. The final medical report submission to FOH and any associated disincentive shall apply in accordance with C.10.2 and C.10.5 from the referral date.

C.9.3 Federal Aviation Administration (FAA) Examinations

A physician certified as an Aviation Medical Examiner (AME) or as a Flight Surgeon must perform Federal Aviation Administration (FAA) examinations. FAA examinations typically consist of a physical evaluation and specific laboratory and testing requirements. The examination cost shall include all required forms and any specimen collection/handling.

A. Commercial Driver's License (CDL) Examinations

Commercial Driver's License (CDL) examinations are used to determine commercial driver physical fitness and must be performed by a CDL – Certified Medical Examiner. The Medical Examiner must have an active listing with the Federal Motor Carrier Safety Administration's National Registry during the time of the exam. CDL examinations typically consist of a physical evaluation and specific laboratory and testing requirements. The examination cost shall include all required forms and any specimen collection and handling.

B. Dental Services

A patient is only considered a "new" patient for the purposes of dental examinations if that patient has never received dental services through the Contractor's PPN. Referrals for dental services will not include any other non-dental services.

C.10 Laboratory Services

The Contractor shall request occupational health laboratory services in accordance with Attachment B.

C.10.1 Laboratory Testing and Analysis

FOH maintains a separate contract with a full-service diagnostic clinical laboratory capable of providing oversight, management, and comprehensive medical laboratory services for all required testing, including, but not limited to those listed in Attachment B – Clinical Laboratories Test List.

The Contractor shall complete all laboratory requisitions necessary to direct examinees to FOH's designated laboratory provider. The FOH-contracted diagnostic clinical laboratory shall furnish the Contractor with the required requisition forms using a method approved by FOH.

Failure by the Contractor to correctly route examinees to FOH's designated diagnostic clinical laboratory provider, including failure to use FOH approved requisition forms or directing examinees to nonauthorized laboratory vendors, shall constitute a material performance deficiency. Each occurrence of incorrect laboratory routing that results in delayed testing, invalid test results, the need for repeat testing, or disruption to FOH clinical operations shall result in the assessment of a financial disincentive in accordance with the Performance Requirements Summary (PRS) or other applicable contract provisions. Repeated failures may result in additional remedies, including but not limited to withholding of payment, reduction of award fee (if applicable), issuance of a Cure Notice, or termination for default as authorized under the terms of this contract.

A physician employed by the Contractor who holds a valid NPI number shall serve as the ordering physician for all clinical laboratory services. The Contractor shall ensure that the ordering physician and all associated physicians adhere to all FOH requirements governing results notification, including any agency-specific notification requirements specified in the work order. The ordering physician is responsible for ordering all laboratory tests, receiving all results, and reporting any critical results to FOH-designated personnel authorized to receive such notifications within one (1) business day of receipt.

FOH may require the Contractor to utilize an alternate method of ordering laboratory test and receiving laboratory test results when circumstances necessitate such a change. Alternate methods may include the use of manual or paper requisition forms and receiving laboratory results via secure fax or overnight courier delivery service.

In the event of any change to the FOH-contracted diagnostic clinical laboratory services or provider during the contract period of performance, FOH will notify the Contractor and coordinate closely to ensure a smooth transition and timely integration of any new or modified diagnostic clinical laboratory service requirements.

When new testing requirements are introduced or existing testing requirements are modified, the FOH COR will provide the Contractor with the necessary information and instructions to support ongoing compliance.

C.10.2 Laboratory Test Requisition Forms

FOH's diagnostic clinical laboratory shall provide the Contractor with paper laboratory requisition forms. Each requisition form shall be pre-printed with the Contractor's address and shall contain FOH's most frequently ordered laboratory tests along with associated test codes. With approval from the COR, requisition forms may also include pre-printed information for the Contractor's referring physician.

The requisition forms shall allow for the manual addition of any diagnostic clinical laboratory test requests not included in the pre-printed test list. FOH's diagnostic clinical laboratory shall supply the Contractor with all materials required to generate requisitions and specimen labels, including any supplies needed that differ from standard letter-sized paper.

FOH reserves the right to transition from paper to electronic requisition forms at any time during the performance period of this contract, and the Contractor shall comply with all FOH direction regarding such transition.

Failure by the Contractor to utilize FOH approved laboratory requisition forms, including failure to use the FOH supplied paper requisitions, use of outdated or altered forms, omission of required preprinted fields, or submission of requisitions not aligned with FOH approved test codes, shall constitute a material performance deficiency. For each instance in which an incorrect, incomplete, or unauthorized requisition form results in delayed testing, misrouted laboratory services, incorrect billing, or the need for repeat testing, shall result in the assessment of a financial disincentive in accordance with the PRS or other applicable contract provisions. Repeated failures may result in additional remedies, including but not limited to withholding payment, reduction of award fee (if applicable), issuance of a Cure Notice, or termination for default as authorized under the terms of this contract.

These remedies may be applied cumulatively as appropriate to the severity, frequency, and operational impact of noncompliance. The Contractor remains fully responsible for ensuring that all laboratory requisitions are prepared, submitted, and routed using only FOH approved requisition forms and in accordance with all instructions provided by FOH.

C.10.3 Laboratory Test Results

The FOH diagnostic clinical laboratory provider shall provide laboratory results to the Contractor using a method approved by FOH. In the event of an interruption to the approved primary method of requesting or delivering results, or disruptions at the diagnostic clinical laboratory processing facility, FOH's diagnostic clinical laboratory will provide an alternative or secondary method to ensure timely resulting.

The ordering physician shall also notify clients of all laboratory testing results, provide appropriate clinical and follow-up recommendations, and ensure all results, discussion and recommendations are accurately documented in the client's medical record. All normal test results shall be communicated to the client within five (5) business days.

The Contractor's ordering physician is responsible for reporting any abnormal or critical medical test or lab results to the client. Critical results along with follow-up instructions must be reported to the client immediately upon receipt and within no more than 24 hours of the lab communicating the critical value to the provider. Abnormal results and follow-up instructions must be reported to the client within 2 business days of the lab communicating the abnormal value to the provider. In addition, the Contractor shall notify FOH of critical labs, including positive TB blood tests, and other specified test results to FOH within 24 hours electronically via FOH designated instructions.

Certain agencies require notification for abnormal results and for post-exposure testing. The Contractor shall ensure all notification instructions provided in the work order are strictly followed. The Contractor shall comply with any agency specific reporting protocols, including timelines and follow up testing standards as outlined in applicable work orders or FOH clinical guidance. In addition, the Contractor shall notify FOH-designated personnel authorized to receive results of all positive Tuberculosis (TB) testing results and all post-exposure TB test results, including both positive and negative results, within 24 hours of receiving the results. The Contractor shall further ensure that all providers adhere to mandatory infectious disease reporting requirements as

established by the state and as required by the Centers for Disease Control and Prevention (CDC).

Failure by the Contractor to notify FOH designated personnel of abnormal clinical results or required postexposure testing results, including, but not limited to, all positive Tuberculosis (TB) test results and all postexposure TB test results (positive and negative), within the required twenty four (24) hour timeframe shall result in the assessment of a financial disincentive in accordance with the Performance Requirements Summary (PRS) or other applicable contract provisions. Repeated failures may result in additional remedies, including but not limited to withholding payment, reduction of award fee (if applicable), issuance of a Cure Notice, or termination for default as authorized under the terms of this contract. These remedies are cumulative and may be applied concurrently based on the severity or recurrence of the violation. The Contractor remains fully responsible for ensuring timely, accurate, and complete reporting of all abnormal and TBrelated results in accordance with FOH requirements, FOH SOPs, and state and CDC infectious disease reporting requirements.

The Contractor shall ensure that all laboratory results received through any approved primary or alternate method are accurately paired with the correct patient referral within the Contractor's secure web portal.

Failure by the Contractor to correctly pair laboratory test results with the appropriate patient referral within the Contractor's secure web portal, as required by FOH, shall constitute a material performance deficiency. This includes, but is not limited to, instances where mis-paired, unpaired, incorrectly assigned, or delayed pairing of laboratory results in:

- Delays in clinical review or report submissions
- Disruption of FOH operations.
- The need for repeat laboratory testing.
- Incomplete or inaccurate final reports submitted to FOH.

For each instance the Contractor fails to properly pair laboratory results shall result in the assessment of a financial disincentive in accordance with the PRS or other applicable contract provisions. Repeated failures may result in additional remedies, including but not limited to withholding of payment, reduction of award fee (if applicable), issuance of a Cure Notice, or termination for default as authorized under the terms of this contract.

These remedies may be applied individually or in combination, based on the severity, frequency, and operational impact of the non-compliance. The Contractor remains fully responsible for ensuring that all laboratory results are promptly, accurately, and completely paired with the correct patient referral in accordance with FOH requirements

All test results must be included in the final report submission to FOH for the referral to be considered complete. The Contractor shall schedule appointments and manage workflows to account for the laboratory test turnaround times to meet required referral submission deadlines. Result turnaround times for all tests available to FOH customers will be provided to the Contractor upon referral. When laboratory turnaround times impact the Contractor's ability to meet the required submission timeframe, the Contractor shall annotate the referral accordingly and submit a referral to the COR for a waiver of any associated report submission penalty.

C.10.4 Laboratory Collection Equipment and Supplies

The FOH contracted diagnostic clinical laboratory service provider shall supply the Contractor with all necessary specimen-collection equipment and supplies, including but not limited to vacutainer tubes and holders, urine specimen containers, secondary specimen containers, and absorbent materials. All equipment and supplies must conform to accepted clinical standards and be suitable for analysis at the primary FOH diagnostic clinical laboratory or an approved alternate laboratory.

All equipment and supplies shall meet current Federal OSHA and applicable State OSHA standards. Materials intended for specimen transport must comply with Department of Transportation (DOT) and International Air Transport Association (IATA) Dangerous Goods Regulations governing the shipment of biological or diagnostic specimens. Adequate insulating containers and synthetic refrigerant packs shall be supplied for temperature sensitive specimens. The FOH diagnostic clinical laboratory service shall provide a mechanism for the Contractor to order and receive additional supplies, and the Contractor shall receive all requested supplies within five (5) business days of submitting the request.

FOH Customized Kits

FOH employs a variety of procedures to meet complex customer requirements for physical examinations and laboratory testing. One such method is the use of FOH customized kits, small boxes or envelopes containing standardized instructions and supplies that are shipped to specified locations via express delivery for use in collection and subsequent shipment of specimens. Each kit includes all required specimen collection devices and containers. Kits are designed to reduce collection errors, standardize laboratory testing, and avoid unnecessary variation in supplies and shipping methods. Completed kits are returned via overnight delivery at FOH expense to the FOH diagnostic clinical laboratory for processing.

The Contractor shall ensure that all required laboratory kits are delivered and available at the provider location no fewer than five (5) business days prior to any scheduled appointments.

In the event the Contractor fails to ensure that required laboratory kits and examination packets are available at the designated provider location no fewer than five (5) business days prior to any scheduled appointment, such failure shall constitute a performance deficiency. For each occurrence in which a scheduled appointment cannot proceed as planned due to kit unavailability, the Government may assess a financial disincentive in accordance with the Performance Requirements Summary (PRS) or other applicable contract provisions. Repeated failures may result in additional remedies, including but not limited to withholding payment, reduction of award fee (if applicable), issuance of a Cure Notice, or termination for default as authorized under the terms of this contract.

The FOH Clinical Laboratory Contractor will provide the kit contents to the Contractor's Program Manager. The type and number of collection containers will be determined by FOH needs and the diagnostic clinical laboratory's methodologies. Any recommended changes to kit contents require 14 calendar days' written notice to the COR.

Kits shall be constructed to maintain specimen stability during transport and shall meet all DOT and IATA Dangerous Goods Regulations. Kits shall contain all required specimen collection devices and containers appropriate for the testing included in the kit. Kits should include the following:

- Gauze pads
- Alcohol wipe
- Biohazard bag with absorbent material
- Vial sleeves
- FOH provided airbills from the overnight delivery service vendor
- Plastic shipping bags from the overnight delivery service carrier

Before using any FOH supplied customized kit, the Contractor shall:

- Verify that all kit contents meet the requirements for the diagnostic clinical laboratory tests specified in the applicable work/service order.
- Insert all necessary diagnostic clinical laboratory instructions and shipping carrier airbills (if required).
- Insert all completed laboratory requisitions.

Failure by the Contractor to properly handle FOH supplied customized laboratory kits or to ensure that each kit contains all required components—including specimen collection devices, containers, biohazard materials, shipping materials, and FOH provided airbills—shall constitute a material performance deficiency. This includes, but is not limited to:

- Use of incomplete kits.
- Omission of required components.
- Improper assembly, storage, or handling of kits.
- Failure to verify kit contents before use.
- Improper preparation of kits for shipment.
- Failure to maintain compliance with DOT and IATA Dangerous Goods Regulations for biological specimens.

For each instance in which improper kit handling or missing components result in delayed testing, compromised specimen integrity, need for recollection, or noncompliance with FOH laboratory procedures, the Government may assess a financial disincentive in accordance with the Performance Requirements Summary (PRS) or other applicable contract provisions. Repeated failures may result in additional remedies, including but not limited to withholding payment, reduction of award fee (if applicable), issuance of a Cure Notice, or termination for default as authorized under the terms of this contract.

The Contractor shall provide the COR with a detailed monthly inventory report identifying the number of customized kits used each month for private provider specimen collections. The Contractor shall utilize the Government preferred shipping carrier for all kit shipments. Associated shipping costs will be reimbursed by the Government and must be included in the Contractor's monthly invoice.

C.11 Storage and Distribution of Government Supplied Medications

C.11.1 Travel Medications

FOHNet work order requirements may include travel medications to be dispensed to FOH customers. Travel medications may include Acetazolamide, Azithromycin, Chloroquine, Ciprofloxacin, Loperamide, Malarone, Mefloquine, Primaquine and Doxycycline. Some of these medications may be supplied to an employee by FOH. The Contractor's providers must conduct a

brief exam, administer indicated vaccines for travel, and prescribe FOH-approved travel medications when needed. The Contractor shall order FOH medications as listed on the FOH formulary or medication approved by FOH and from an FOH approved distributor. FOH will provide instructions for ordering after contract award.

C.11.2 Vaccines

FOHNet work order requirements may include immunization administration of vaccines. Some vaccines may be difficult to obtain. The Contractor shall obtain approval from the COR to order vaccines from an FOH approved distributor. FOH will provide a list of vaccines available for order through an FOH approved distributor and instructions for ordering after contract award. When administering a vaccine that was obtained at the expense of the government from an FOH approved distributor, the Contractor shall only charge FOH using the appropriate Immunization Administration service line item(s) in *Attachment A*.

The Contractor will be expected to store and maintain a stock of all vaccines listed in *Attachment A* to distribute to providers for administration, when needed. The Contractor shall follow vaccine storage and handling best practice guidelines provided by the Centers for Disease Control and Prevention (CDC).

All vaccine line items listed in *Attachment A* shall include both the cost of the vaccine and the administration of the vaccine. In some instances, a brief examination may be required by a provider prior to administering a particular vaccine. These instances must be pre-approved by the COR and shall not apply to influenza vaccine administration.

C.11.3 Written Prescriptions (Ad-Hoc)

FOHNet work orders may require a licensed provider to provide a patient with a written prescription that may be filled by any pharmacy later.

C.12 Credentialing and Privileging Process

The Contractor shall manage and administer all aspects of the PPN in compliance with applicable The Joint Commission (TJC) Ambulatory Care Human Resource standards, including the establishment and maintenance of complete, accurate, and up-to-date credentialing and privileging files for all PPN providers.

The Contractor's credentialing process for each PPN provider shall include, at a minimum:

- Primary source verification of all active professional licenses from an industry-recognized primary-source verification system.
- Verification of education and training, including academic degrees, professional training, residencies, and board-certifications.
- Background check.
- National Practitioner Data Bank (NPDB) query.
- Verification of federal exclusion status, including:
 - HHS Office of the Inspector General (OIG) Exclusions Database
 - General Services Administration (GSA) System for Award Management Exclusions

Database

- Verification of required US Department of Transportation certifications, when the provider conducts Commercial Driver's License (CDL) or Federal Aviation Administration (FAA) examinations

In alignment with TJC Ambulatory Care standards, credentialing verifies a provider's qualifications and professional standing, while privileging establishes the specific clinical services and procedures the provider is authorized to perform. The Contractor must maintain supporting documentation to justify that a provider has the appropriate education, experience and skill to provide a FOH requested service.

If any sanctions, censures, or adverse findings are identified, and the findings do not automatically disqualify the provider, the Contractor shall submit complete supporting documentation to the COR for review per FOH policies. FOH retains final authority to determine a provider's eligibility for participation in the PPN.

All PPN provider credentials shall undergo full reverification every 3 years using the credentialing steps listed above. At least annually, the Contractor shall provide the COR with a written report certifying that that each LIP's NPDB query has not contained any reported items during the past year.

C.13 Fees

The fees below will be invoiced using the Other Direct Cost (ODC) contract line item.

C.13.1 Missed Appointment (No-Show) and Late Cancellation Fee

FOH PPN referrals (i.e., diagnostics, exams, etc.) shall include a provision for a missed appointment (no show)/late cancellation fee. This fee can be charged for failure to appear, reschedule, or cancel a scheduled provider appointment within 24-hours of the scheduled appointment time. No fee shall exceed \$50.00.

Psychological services may be subject to two (2) separate missed appointment (no show)/late cancellation fees when 2 services are required to complete a referral. For example, one referral can include a psychological pre-employment and security clearance exam. This fee can be charged for failure to appear or reschedule. The price of these fees shall be renegotiated prior to each option period.

C.13.2 Out of Network Fee

The Out of Network fee will be utilized when an alternative provider site is requested, although a qualified private provider within the PPN is contracted and available. The Contractor can only bill this fee for the initial referral appointment using the provider site, not for each employee/applicant appointment with the same provider. The fee is based on the cost incurred by the Contractor in setting up a new network provider. If FOH requires service by an Out of Network service provider due to In-network provider performance issues, then the fee shall not be applied.

C.13.3 Expedited Service Fee

The expedited fee will be utilized when the referral clearly indicates that expedited service is required. The fee will be billed upon completion of all work order requirements. The fee is intended to cover the cost incurred by the Contractor to accelerate normal business practices. Failure to meet the timeframes specified in C.10.3 and/or C.10.6 on an expedited referral will result in the forfeiture of the expedited fee, and the Standard Referral Service Completion (C.10.2) requirements and/or Standard Referral Report Submission (C.10.5) requirements shall apply. The expedited service option is intended for applicant/pre- placement exams, with the exception of urgent customer employee deployments, exit exams, or other special circumstances requested by the COR. This service may not be available to all FOH customers. The government anticipates expedited service referrals will not exceed 15% of all FOHNet referrals on an annual basis.

C.13.4 Preferred Provider Fee

The preferred provider fee is applicable when FOH directs the use of a specific provider, on behalf of FOH customers, that is not a member of the private provider network at the time of the referral. This fee is designed to cover additional costs when negotiations with the preferred provider have resulted in rates that exceed the rates found in *Attachment A*. The contractor shall furnish the COR with documentation demonstrating the additional costs. The supplemental fee may be billed in addition to the charges normally incurred for requested services and charged for each client visit to the preferred provider site. The COR must pre- approve the use of all preferred provider sites. Once approved by FOH, the site will maintain the preferred provider designation for the remainder of the contract.

C.14 Contractor Self-Assessment and Monthly Workplan

The Contractor shall provide to the COR with each monthly invoice a self-assessment of the Contractor's performance in the billing period for which payment is requested, which will include at a minimum:

- Section C.19 (Surveillance Matrix) and F.3 (Deliverables Chart) Line Number.
- Services performed outside the mileage requirement.
- New Provider status updates.
- Listing of records that were unable to be scheduled due to availability.
- Applicable disincentives – cumulative disincentives for a single referral shall not exceed the total cost of services for that referral.
- Challenges encountered and successes realized.
- Contractor's plan(s) to correct causes of disincentives.
- PPN Provider Site Detail failing to meet all requirements in the Work order (i.e., Missing Items).

Failure to include the Self-Assessment Report with the monthly invoice will result in the rejection of the invoice. FOH data associated with services final report submissions will be available to the Contractor upon request.

In addition to monthly self-assessments, the Contractor will provide an updated work plan each month showing service units completed in the previous month and those anticipated for the coming month(s). Should surges or dips in service demand be anticipated, these monthly updates will serve to alert the Contractor or the government, whichever party has the requisite information, as to the

change in expected service demand. These service units will be tracked against a baseline work plan submitted to the COR within 2 weeks of each option or task order executed, for which an updated work plan would be required.

In-Progress Reviews (IPRs) The Contractor shall arrange, facilitate and execute monthly IPRs, as well as any additional IPRs at the request of the COR, during the first year of the contract. After the first year of the contract, the IPRs will be assessed to determine continuation of monthly IPRs or quarterly IPRs, at the discretion of the government. The IPR's will be help in the Microsoft Teams platform and may be recorded for quality assurance. IPRs will be conducted virtually. The Contractor shall be responsible for providing agenda, presentation materials, and meeting minutes for all IPRs. Minimum content for IPRs shall include, but not limited to, the following:

- Overall Contract execution, management and operating status update(s)
- Discussion, as necessary of any upcoming changes, including additions, modifications, deletions
- Recommendation to better optimize contractor operations, optimize communications, with projection for implantation of recommendations
- Issue/Complaint tracking, to include but not limited to identifying complaint, complaint status, complaint resolution, complaint monitoring.
- Improper/Misdirected Billings pending
- Reports requested by the government, to include but not limited to: contract required reports, percentages of work orders processed by month, timeliness of processing, issue percentages by customer and overall contract, and utilization by procedure and customer.
- Discussion of missing exam items.
- Adherence to work orders.
- Any government agenda item provided to the contractor for inclusion in the scheduled IPRs.

Complaint Process

The contractor shall develop a complaint process that includes, complaint in-take, investigation, progress, resolution and corrective action taken to prevent further re-occurrence, if at fault of the Contractor. The Complaint Process plan will be presented to the government for approval before implantation. The plan should include response times and notification contractor will respond to urgent complaints within 2 hours and non-urgent complaints within 2 working days, acknowledging receipt and the plan of action taken. The contractor will present a detailed report at the IPR meetings.

C.15 Improper/Misdirected Billing

The Contractor shall be responsible for ensuring FOH (including the individual employee/applicant) does not receive invoices/bills directly from the Contractor's PPN providers. In the event improper billing is discovered, FOH will inform the Contractor of the error immediately. The Contractor shall resolve the error within ten (10) business days and provide the COR with written proof that the situation has been resolved. Written proof is specified as a zero-balance statement from the responsible Contractor PPN provider billing department absolving FOH, the FOH customer agency, and FOH customer agency employee of any responsibility to the COR, a notarized letter stating that the charges are the sole responsibility of the Contractor, or

another form of written proof pre-approved by the COR. The Contractor must resolve improper billing issues and provide written proof of resolution to the COR within five (5) business days of the bill resolution referral notification to the Contractor from the COR. Failure to resolve improper billing issues within five (5) business days will result in a disincentive of \$25.00 for each business day beyond the five (5) business days.

If it is determined that the service for which the improper/misdirected bill was not performed under the PWS of this contract, the Contractor shall bring these occurrences to the COR's attention so that corrective actions can be taken to update/improve the referral process (on either the Contractor's or the Government's side) so as to minimize or eliminate the occurrence of future improper/misdirected billing.

C.16 FOH Medical Documentation

The work order shall list the required medical documentation needed for each service referral. The Contractor will have access to all required forms and documents. Any FOH customer agency specific (non-FOH standard) medical forms will be provided to the Contractor, by FOH. The Contractor shall be responsible for clerical costs necessary for printing, copying, and collating of all forms and documents used by the Contractor PPN providers. Blank forms can be electronically sent directly to Contractor PPN providers by the Contractor.

The contractor shall establish an expedited (1 day) quality review of all completed forms and medical documentation, required for upload into the PPN. The quality review shall require a 99% accuracy rate and will be part of the quality assurance surveillance plan review in monitoring of the contract.

The forms completion fee is to be used when the PPN practitioner is required to complete an excess number of forms, beyond what can be reasonably completed in the time allotted for the examination. This fee must be approved by the COR prior to use.

Quality of Physical Examination

The Contractor shall ensure that examination reports submitted by their contracted providers are clear, legible and able to be read. The contracted providers shall also include the following in the physical examination.

- Complete and thorough review of past medical history, review of systems and appropriate documentation of pertinent positives and negatives,
- Complete physical exam to include all required components and pertinent positive/negatives,
- Complete review of all testing (including but not limited to labs, audiometry, vision, spirometry, electrocardiogram, as indicated), and
- Follow-up recommendations as indicated.

Audiometry

In addition, the providers shall ensure Audiometers are calibrated according to OSHA standards and audiometry testing follows standards set forth by OSHA in accordance with work order

requirements set forth by FOH.

Spirometry

The Contractor shall also ensure providers perform spirometry in accordance with the American Thoracic Society's latest standards.

Travel Examinations

The Contractor shall ensure that the provider performs travel examinations in accordance with FOH policies and procedures and in accordance with the Centers for Disease Control and Prevention recommendations.

C.17 Quality Assurance Surveillance Plan (QASP)

The Government intends to utilize a Quality Assurance Surveillance Plan (QASP) to monitor the quality of the Contractor's performance. The oversight provided for in the contract and in the QASP will help to ensure that service levels reach and maintain the required levels throughout the contract term. Further, the QASP provides the COR with a proactive way to avoid unacceptable or deficient performance and provides verifiable input for the Contractor Performance Assessment Reporting System (CPARS). The QASP may be updated by modification to the contract. The Contractor's performance will be subject to both scheduled and unscheduled reviews by the Government, including staff, and customer satisfaction. FOH may at any time monitor and review the Contractor's performance for compliance to FOH policies, standard operating procedures (SOPs), The Joint Commission (TJC) Ambulatory Care (AC) standards and other elements within this performance contract. The Government may unilaterally increase quality review in the event of deficient Contractor performance. In addition, the COR will monitor and evaluate the Contractor's progress through monthly invoices, program reviews, and regular meetings with the Contractor. No penalties shall be assessed during the 60-day transition period; post-transition period penalties shall be assessed at the COR's discretion.

The Surveillance Matrix is a list of performance objectives and standards that must be performed by the contractor. The Surveillance Matrix details the methods of surveillance the COR will use to validate and inspect these performance elements. Performance objectives define the desired outcomes. Performance standards define the level of service required under the contract to successfully meet the performance objective. The inspection methodology defines how, when, and what will be assessed in measuring performance. The Government performs surveillance to determine the quality of the contractor's performance as it relates to the performance element standards.

Ref Para	Performance Objective	Source	Meets Standards	Does Not Meet Standards	Methods of Surveillance / Reporting
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C.8.1	Distance Requirements	Contractor Self-Assessment (C.16)	All services provided per patient for any service within the mileage requirement	Any service performed outside the mileage requirement without written approval (waiver) from the COR will be assessed a charge of up to 20% of all services provided per patient and charged to the Contractor via invoice deduction	COR will assess Contractor Self-Assessment report details submitted with invoice and direct customer notification
C.8.2	New Provider Locations	Contractor Self-Assessment (C.16)	New provider is established	New provider is not established within 30 calendar days of notification from the FOH; will be assessed a penalty of \$2500.00 for each calendar day beyond 30 calendar days and charged to the Contractor via invoice deduction	Initial notification to Contractor from the COR and subsequent Contractor Self-Assessment report details submitted with invoice
C.10.2	Standard Referral Service Completion	Contractor Self-Assessment (C.16)	All services scheduled and completed within 10 business days from referral date	All services are not scheduled and completed within 10 business days from referral; will be assessed a penalty of \$25 500.00 for each business day beyond the 10 business days via invoice deduction.	Contractor Self-Assessment report details submitted with invoice
C.10.3	Expedited Referral Service Completion	Contractor Self-Assessment (C.16)	All services scheduled and completed within 5 business days from referral date	All services are not scheduled and completed within five (5) business days from referral; will result in forfeiture of the Expedited Service Fee and the Standard Referral Service Completion (C.10.2)	Contractor Self-Assessment report details submitted with invoice

				requirements shall apply.	
C.10.5	Standard Referral Report Submission	Contract or Self-Assessment (C.16)	Submit complete report to FOH within 5 business days of service completion date	Complete report is not submitted within five (5) business days of service completion date; will be assessed a penalty of \$500.00 for each business day beyond the ten (10) business days via invoice deduction	Contractor Self-Assessment report details submitted with invoice
C.10.6	Expedited Referral Report Submission	Contract or Self-Assessment (C.16)	Submit complete report to FOH within two (2) business days of service completion date	Complete report is not submitted within two (2) business days of service completion date; will result in forfeiture of the Expedited Service Fee (C.15.4) and the Standard Referral Report Submission (C.10.5) requirements shall apply	Contractor Self-Assessment report details submitted with invoice
C.10.7	Missing Items	Contract or Self-Assessment (C.16)	Submit complete report containing no missing items	Report is submitted that contains one or more missing items; will be assessed a penalty of \$500.00 per missing item via invoice deduction	Contractor Self-Assessment report details submitted with invoice

C.10	Audiometry Requirements	Contractor Self Assessment	Audiometric equipment and calibration must be done according to OSHA standards and in accordance with requirements set forth by the work order/service order.	When Audiometry is scheduled, the Audiometry equipment must be operational, calibrated the audiometry examination completed in accordance with OSHA standards and FOH requirements. Any provider who is not prepared or is unable to provide the Audiometry service without notifying the examinee at least 48 hours in advance, will incur a \$1,000.00 penalty per rescheduled case.	Contractor or FOH Report
C.11.2	Notification of Lab Results (normal or abnormal)	Laboratory Testing and Analysis	Examinee must be notified within 5 business days upon provider's notification of lab results	Failure to notify examinee of lab results within 5 business days will result in a penalty of \$25 for each day of failure to notify	

C.18 Transition

C.18.1 Contractor Transition-In

The transition period will begin upon issuance of the initial task order. The Contractor will have sixty (60) calendar days to execute a transition of operational responsibilities from the incumbent (outgoing) Contractor.

At the end of the sixty (60) calendar day period, the Contractor shall have full operational accountability of all contract requirements.

C.18.2 Contractor Transition-Out

All services shall be transitioned from the incumbent Contractor to the successor Contractor within thirty(30) calendar days.

C.19 Baseline Security Requirements

C.19.1 Applicability

The requirements herein apply whether the entire contract or order (hereafter “contract”), or portion thereof, includes either or both of the following:

A. Access (Physical or Logical) to Government Information

A Contractor (and/or any subcontractor) employee will have or will be given the ability to have, routine physical (entry) or logical (electronic) access to government information.

B. Operate a Federal System Containing Information

A Contractor (and/or any subcontractor) will operate a federal system and information technology containing data that supports the HHS mission. In addition to the Federal Acquisition Regulation (FAR) Subpart 2.1 definition of “information technology: (IT), the term as used in this section includes computers, ancillary equipment (including imaging peripherals, input, output, and storage devices necessary for security and surveillance), peripheral equipment designed to be controlled by the central processing unit of a computer, software, firmware and similar procedures, services (including support services), and related resources.

C.19.2 Safeguarding Information and Information Systems

In accordance with the Federal Information Processing Standards Publication (FIPS) 199, *Standards for Security Categorization of federal Information and Information Systems*, the Contractor (and/or any subcontractor) shall:

A. Protect government information and information systems in order to ensure:

- Confidentiality, which means preserving authorized restrictions on access and disclosure, based on the security terms found in this contract, including means for protecting personal privacy and proprietary information;
- Integrity, which means guarding against improper information modification or destruction, and ensuring information non-repudiation and authenticity; and
- Availability, which means ensuring timely and reliable access to and use of information.

B. Provide security for any Contractor systems, and information contained therein, connected to an HHS network or operated by the Contractor on behalf of HHS regardless of location. In addition, if new or unanticipated threats or hazards are discovered by either the agency or contractor, if existing safeguards have ceased to function, the discover shall immediately, within one (1) hour or less, bring the situation to the attention of the other party.

C. Adapt and implement the policies, procedures, controls, and standards required by the HHS Information Security Program to ensure the confidentiality, integrity, and availability of government information and government information systems for which the Contractor is responsible under this contract or to which the Contractor may otherwise have access under this contract. Obtain the HHS Information Security Program security requirements, outlined in the HHS

Information Security and Privacy Policy (IS2P), by contacting the COR and emailing fisma@hhs.gov.

C.19.3 Information Security Categorization

In accordance with FIPS 199 and National Institute of Standards and Technology (NIST) Special Publication (SP) 800-60, *Volume II: Appendices to Guide for Mapping Types of Information and Information Systems to Security Categories*, Appendix C, and based on information provided by the ISSO, CISO, or other security representative, the risk level for each Security Objective and the Overall Risk Level, which is the highest watermark of the three factors (Confidentiality, Integrity, and Availability) of the information or information system are the following:

- Confidentiality: MODERATE
- Integrity: MODERATE
- Availability: MODERATE
- Overall Risk Level: MODERATE

Based on information provided by the ISSO, Privacy Office, system/data owner, or other security or privacy representative, it has been determined that this solicitation/contract involves PII.

Personally Identifiable Information (PII). Per the Office of Management and Budget (OMB) Circular A-130, "PII is information that can be used to distinguish or trace an individual's identity, either alone or when combined with other information that is linked or linkable to specific individual," Examples of PII include, but are not limited to the following: social security number, date and place of birth, mother's maiden name, biometric records, etc.

- PII Confidentiality Impact Level has been determined to be: MODERATE

C.19.4 Controlled Unclassified Information (CUI)

CUI is defined as "information that laws, regulations, or Government-wide policies require to have safeguarding or dissemination controls, excluding classified information." The Contractor (and/or any subcontractor) must comply with *Executive Order 13556, Controlled Unclassified Information, (implemented at 3 CFR, part 2002)* when handling CUI. 32 CFR 2002.4(aa) as implemented the term "handling" refers to "...any use of CUI, including but not limited to marking, safeguarding, transporting, disseminating, reusing, and disposing of the information." 81 Fed Reg 63323. All sensitive information that has been identified as CUI by a regulation or statute, handled by this solicitation/contract, shall be:

- marked appropriately;
- disclosed to authorized personnel on a Need-To-Know basis;
- protected in accordance with NIST SP 800-534, *Security and Privacy Controls for Federal Information Systems and Organizations* applicable baseline if handled by a Contractor system operated on behalf of the agency, or NIST SP 800-171, *Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations* if handled by internal Contractor systems; and
- returned to HHS control, destroyed when no longer needed, or held until otherwise directed. Destruction of information and/or data shall be accomplished in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.

C.19.5 Protection of Sensitive Information

For security purposes, information is or may be sensitive because it requires security to protect its confidentiality, integrity, and/or availability. The Contractor (and/or any subcontractor) shall protect all government information that is or may be sensitive in accordance with OMB Memorandum M-06-16, *Protection of Sensitive Agency Information* by security it with a FIPS 140-2 validated solution.

C.19.6 Confidentiality and Nondisclosure of Information

Any information provided to the contractor (and/or any subcontract) by HHS or collected by the contractor on behalf of HHS shall be used only for the purpose of carrying out the provisions of this contract and shall not be disclosed or made known in any manner to any persons except as may be necessary in the performance of the contract. The Contractor assumes responsibility for protection of the confidentiality of Government records and shall ensure that all work performed by its employees and subcontractors shall be under the supervision of the Contractor. Each Contractor employee or any of its subcontractors to whom HHS records may be made available or disclosed shall be notified in writing by the Contractor that information disclosed to such employee or subcontract can be used only for that purpose and to the extent authorized herein.

The confidentiality, integrity, and availability of such information shall be protected in accordance with HHS and FOH policies. Unauthorized disclosure of information will be subject to the HHS/FOH sanction policies and/or governed by the following laws and regulations:

- 18 USC 641 (Criminal Code: Public Money, Property or Record);
- 18 USC 1905 (Criminal Code: Disclosure of Confidential Information); and
- 44 USC, Chapter 35, Subchapter I (Paperwork Reduction Act).

C.19.7 Internet Protocol Version 6 (IPv6)

All procurements using Internet Protocol shall comply with OMB Memorandum M-05-22, *Transition Planning for Internet Protocol Version 6 (IPv6)*.

C.19.8 Government Websites

All new and existing public-facing government websites must be securely configured with Hypertext Transfer Protocol Secure (HTTPS) using the most recent version of Transport Layer Security (TLS). In addition, HTTPS shall enable HTTP Strict Transport Security (HSTS) to instruct compliant browsers to assume HTTPS at all times to reduce the number of insecurity redirects and protect against attacks that attempt to downgrade connections to plain HTTP. For internal-facing websites, the HTTPS is not required, but it is highly recommended.

C.19.9 Contract Documentation

The Contractor shall use provided templates, policies, forms and other agency documents provided by FOH to comply with contract deliverables as appropriate.

C.19.10 Standard for Encryption

The Contractor (and/or any subcontractor) shall:

- A. Comply with the *HHS Standard for Encryption of Computing Devices and Information* to prevent unauthorized access to government information. Encrypt all sensitive federal data and information (i.e., PII, protected health information (PHI), proprietary information, etc.) in transit (i.e., email, network connections, etc.) and at rest (i.e., servers, storage devices, mobile device, backup media, etc.) with FIPS 140-2 validated encryption solution.
- B. Secure all devices (i.e. desktops, laptops, mobile devices, etc.) that store and process government information and ensure devices meet HHS and FOH-specific encryption standard requirements. Maintain a complete and current inventory of all laptop computers, desktop computer, and other mobile devices and portable media that store or process sensitive government information (including PII).
- C. Verify that the encryption solutions in use have been validated under the Cryptographic Module Validation Program to confirm compliance with FIPS 140-2. The Contractor shall provide a written copy of the validated documentation to the COR upon referral.
- D. Use the Key Management system on the HHS personal identification verification (PIV) card or establish and use a key recovery mechanism to ensure the ability for authorized personnel to encrypt/decrypt information and recover encryption keys. Encryption keys shall be provided to the COR upon referral and at the conclusion of the contract.

C.19.11 Contractor Non-Disclosure Agreement (NDA)

Each Contractor (and/or any subcontractor) employee having access to non-public government information under this contract shall complete the FOH non-disclosure agreement, as applicable. A copy of each signed and witnessed NDA shall be submitted to the Contracting Officer (CO) and/or COR prior to performing any work under this acquisition.

C.19.12 Privacy Threshold Analysis (PTA)/Privacy Impact Assessment (PIA)

The Contractor shall assist OS Senior Official for Privacy (SOP) or designee with conducting the PTA for the information system and/or information handled under this contract to determine whether or not a full PIA needs to be completed.

- A. If the results of the PTA show that a full PIA is needed, the Contractor shall assist the OS SOP or designee with completing a PIA for the system or information after completion of the PTA and in accordance with HHS policy and OMB M-03-22, *Guidance for Implementing the Privacy Provisions of the E-Government Act of 2002*.
- B. The Contractor shall assist the OS SOP or designee in reviewing the PIA at least every three years throughout the system development lifecycle (SDLC)/information lifecycle, or when determined by the agency that a review is required based on a major change to the system, or when new types of PII are collected that introduces new or

increased privacy risks, whichever comes first.

C.19.13 NIST Compliance

The Contractor shall ensure that the security for the systems supported under this contract is compliant with the protocols defined in NIST Special Publication 800.53 rev 3 or updated revisions. The National Institute of Standards and Technology (NIST) Special Publications (SP) 800-53 Rev. 3 or newer, NIST SP 800-53A Rev. 1, and NIST 800-37 Rev. 1 establishes guidance and requirements for conducting security authorizations and introduces the concept of a Risk Management Framework (RMF). NIST 800.53 security standards can be accessed at <http://csrc.nist.gov/groups/SMA/fisma/assessment.html> or <http://csrc.nist.gov/publications/nistpubs/800-53A-rev1/sp800-53A-rev1-final.pdf>.

System owners are responsible for addressing NIST SP 800-53 Rev. 3 in their implementations and for developing the security artifacts that address NIST SP 800-53 Rev. 3 controls. Authorization and security assessment efforts shall be mapped to NIST SP 800-53 Rev. 3 and reflect what controls guidance are implemented in a system.

Authorization to Operate (ATO) decisions will also follow these standards with approval decisions based on compliance with NIST SP 800-53 Rev. 3 control requirements. The Contractor must receive an Authorization to Operate (ATO) from the HHS Chief Information Officer, for the systems that will be used to manage sensitive data for the Government.

The scope of this accreditation effort is the contractor system(s) storing FOH health information and the associated documentation necessary to fulfill the NIST security requirements. For example, motherboard microcode and email systems are outside the scope; however, co-mingled web and application servers are considered within the boundaries of the FOH NIST accreditation. To the extent possible, the Contractor will provide access to information documents or supporting artifacts prior to the ST&E. Some documentation may only be released during the physical on site evaluation by HHS/OCIO representatives. The Contractor will be responsible for executing system scans based on software configuration parameters provided by HHS/OCIO. Non-contractor employees will not be permitted to attach non-contractor assets to the Contractor computer network. The Contractor will transmit the system scan parameters and results to the HHS/OCIO designee for further analysis and approval. The Contractor shall host all IT systems at a facility that meets NIST 800.53 rev 3 security standards. The Contractor shall submit to on-sight inspections on an annual basis to allow the Government to validate that the site is being maintained in a compliant manner that is supportive of the documented security plan. The Government shall coordinate these visits with at least 30 days' notice.

C.20 Security Training

C.20.1 Mandatory Training for All Contractor Staff

All Contractor (and/or subcontractor) employees assigned to work on this contract shall complete the applicable HHS/FOH Contractor Information Security Awareness, Privacy, and Records Management training (provided upon contract award) before performing any work under this contract. Thereafter, the employees shall complete FOH Information Security Awareness, Privacy, and Records Management training at least annually, during the life of this contract. All provided training shall be compliant with HHS training policies.

C.20.2 Role-based Training

All Contractor (and/or subcontractor) employees with significant security responsibilities (as determined by the Program Manager) must complete role-based training annually commensurate with their role and responsibilities in accordance with HHS policy and the *HHS Role-Based Training (RBT) of Personnel with Significant Security Responsibilities Memorandum*.

C.20.3 Training Records

The Contractor (and/or any subcontractor) shall maintain records for all its employees working under this contract in accordance with HHS policy. A copy of the training records shall be provided to the COR within 30 days after contract award and each subsequent task order thereafter, or upon referral.

C.21 Security Rules of Behavior

C.21.1 The Contractor (and/or any subcontractor) shall ensure that all employees performing on the contract comply with the *HHS Information Technology General Rules of Behavior*, and any FOH policies as applicable.

C.21.2 All Contractor employees performing on the contract must read and adhere to the Rules of Behavior before accessing Department data or other information, systems, and/or networks that store/process government information, initially at the beginning of the contract and at least annually thereafter, which may be done as part of annual FOH Information Security Awareness Training. The Contractor shall submit to the COR a signed Signature Page found in the form for each employee prior to employees performing any work under this contract and annually thereafter.

C.22 Security Incident Response

The Contractor (and/or any subcontractor) shall respond to all alerts/Indicators of Compromise (IOCs) provided by HHS Computer Security Incident Response Center (CSIRC)/FOH IRT teams within 24 hours, whether the response is positive or negative.

Federal Information Security Modernization Act (FISMA) defines an incident as “an occurrence that (1) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information or an information system; or (2) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable use policies. The *HHS Policy for IT Security and Privacy Incident Reporting and Response* further defines incidents as events involving cybersecurity and privacy threats, such as viruses, malicious user activity, loss of, unauthorized disclosure or destruction of data, and so on.

A privacy breach is a type of incident and is defined by FISMA as the loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where:

(1) a person other than an authorized user accesses or potentially accesses personally identifiable information or (2) an authorized user accesses or potentially accesses personally identifiable information for an other than authorized purpose. The *HHS Policy for IT Security and Privacy Incident Reporting and Response* further defines a breach as “a suspected or confirmed incident involving PII.”

In the event of a suspected or confirmed incident or breach, the Contractor (and/or any subcontractor) shall:

C.22.1 Protect all sensitive information, including any PII created, stored, or transmitted in performance of this contract so as to avoid a secondary sensitive information incident with FIPS140-2 validated encryption.

C.22.2 NOT notify affected individuals unless so instructed by the Contracting Officer or COR. If so, instructed by the Contracting Officer or COR, the Contractor shall send FOH recommended notifications to affected individuals for approval.

C.22.3 Report all suspected and confirmed information security and privacy incidents and breaches to the FOH Incident Response Team (IRT), COR, CO, OS SOP (or his/her designee), and other stakeholders, including incidents involving PII, in any medium or form, including paper, oral, or electronic, as soon as possible without unreasonable delay, no later than one (1) hour, and consistent with the applicable FOH and HHS policy and procedures, NIST standards and guidelines, as well as US-CERT notification guidelines. The types of information required in an incident report must include at a minimum company and point of contact information, contract information, impact classifications/threat vector, and the type of information compromised. In addition, the Contractor shall:

- cooperate and exchange any information, as determined by the Agency, necessary to effectively manage or mitigate a suspected or confirmed breach;
- not include any sensitive information in the subject or body of any reporting e-mail; and
- encrypt sensitive information in attachments to email, media, etc.

C.22.4 Comply with OMB M-17-12, *Preparing for and responding to a Breach of Personally Identifiable Information* HHS/FOH incident response policies when handling PII breaches.

C.22.5 Provide full access and cooperate on all activities as determined by the Government to ensure an effective incident response, including providing all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. This may involve disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks, or applying additional security controls. This may also involve physical access to contractor facilities during a breach/incident investigation.

C.23 Position Sensitivity Designations

All Contractor (and/or any subcontractor) employees must obtain a background investigation commensurate with their position sensitivity designation that complies with Parts 1400 and 731 of Title 5, Code of Federal Regulations (CFR). To perform the work specified herein, Contractor employees shall require access to sensitive data, regular access to HHS-controlled facilities and/or access to HHS information systems. The Government has determined the position sensitivity under this effort to be either Tier 2 or Tier 4. The following position sensitivity designation levels apply to this solicitation/contract:

- The Government anticipates the position sensitivity under this effort to be: Tier 2(MBI) for access to PII and Tier 4 (BI) for Elevated/Privileged Access (IT Sys Admin); however, sensitivity levels cannot be ascertained definitively until after contract award.
- The minimum Government investigation to receive an HSPD-12 PIV Credential is a Tier 1

(NACI) with fingerprints, which consists of searches of records covering specific areas of a person's background during the past five years. Those inquiries are sent to current and past employers, schools attended, references, and local law enforcement authorities. More restricted positions, above non-sensitive, require more extensive documentation and investigation.

- The minimum Government investigation to receive Elevated Privilege (any access beyond normal email or data entry) is a Tier 4 (BI High Risk Public Trust).

C.23.1 The contractor must comply with the instructions and timeframes provided by the Contracting Officer's Representative (COR) regarding the handling of the security requirements specified in this section. Typically, each employee must submit at a minimum: a completed OF-306; a current resume; and a completed HHS 828 form. Additional requirements may apply to Foreign National Applicants who will follow a slightly different level of initial checks. No contractor may begin any work or obtain any access to HHS facilities, information or information systems unless and until codified credentialing standards are met, and the Office of Security and Strategic Information informs the COR initial security requirements are complete or HSPD-12 badges have been issued, whichever occurs first. Contractors should ensure that the employees whose names they submit have a reasonable chance for access approval. In some cases, employees with existing background investigations commensurate with sensitivity designations will expedite performance. Delays associated with rejections and consequent reinvestigations may not be excusable. Investigations may delay performance, regardless of the outcome of the investigation. The facts surrounding individual cases will determine whether a delay is excusable.

U.S. law requires companies to employ only individuals who may legally work in the United States – either U.S. citizens, or foreign citizens who have the necessary authorization. The Contractor shall use e-Verify, an electronic system operated by the U.S. Citizenship and Immigration Service and the Social Security Administration, to ensure Contractor personnel are legal to work under this contract.

C.23.2 Typically, the Government investigates personnel at no cost to the contractor, but the expense of multiple investigations for the same position is difficult to justify.

C.24 Homeland Security Presidential Directive (HSPD)-12

The Contractor (and/or any subcontractor) and its employees shall comply with Homeland Security Presidential Directive-12, *Policy for a Common Identification Standard for Federal Employees and Contractors*; OMB M-05-24, FIPS 201, *Personal Identity Verification (PIV) of Federal Employees and Contractors*; HHS HSPD-12 policy; and Executive Order 13467, Part 1 at 1.2.

C.25 Contract Security Initiation and Expiration

C.25.1 General Security Requirements

The Contractor (and/or any subcontractor) shall comply with information security and privacy requirements, Enterprise Performance Life Cycle (EPLC) processes, HHS Enterprise Architecture requirements to ensure information is appropriately protected from initiation to expiration of the contract. All information systems development or enhancement tasks supported by the contractor shall follow the HHS EPLC framework and methodology and in accordance with the HHS Contract Closeout Guide (2012).

C.25.2 System Documentation

Contractors (and/or any subcontractor) must follow and adhere to NIST SP 800-64, *Security Considerations in the System Development Life Cycle*, at a minimum for system development and provide system documentation at designated intervals (specifically at the expiration of the contract) within the EPLC that require artifact review and approval.

C.25.3 Sanitization of Government Files and Information

As part of contract closeout and at expiration of the contract, the Contractor (and/or any subcontractors) shall provide all required documentation to the COR to certify that, at the government's discretion, all electronic and paper records are appropriately disposed of and all devices and media are sanitized in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.

C.25.4 Notification

The Contractor (and/or any subcontractor) shall notify the COR and system ISSO within FOH before an employee stops working under this contract.

C.25.5 Contractor Responsibilities upon Physical Completion of the Contract

The Contractor (and/or any subcontractor) shall return all government information and IT resources (i.e., government information in non-government-owned systems, media, and backup systems) acquired during the term of this contract to the COR. Additionally, the Contractor shall provide a certification that all government information has been properly sanitized and purged from Contractor-owned systems, including backup systems and media used during contract performance, in accordance with HHS and/or FOH policies.

C.25.6 The Contractor (and/or any subcontractor) shall perform and document the actions identified in FOH Contractor Employee Separation Checklist when an employee terminates work under this contract within three (3) business days of the employee's exit from the contract. All documentation shall be made available to the COR upon referral.

C.26 Operation & Maintenance ATO (Authority to Operate)

It has been determined the contract is subject to the requirements of an ATO and the Contractor shall obtain and maintain an ATO for the duration of this contract. The Contractor shall submit any renewal as per the requirements of FOH Information System Security Office (ISSO) at least 45 days in advance.

The following, but not limited to, shall be included in the ATO submission package:

- 1) Security Assessment Plan (SAP)
- 2) System Security Plan (SSP)
- 3) Enterprise Mission Assurance Support Service (eMASS) records
- 4) Scan reports - provide ACAS vulnerability, MECM, and other applicable requested scans for each instance implemented
- 5) Plan of Action & Milestones (POA&Ms)
- 6) Ports, Protocols, and Services (PPS)

- 7) Incident Response Plan (IRP)
- 8) Information System Contingency Plan (ISCP)
- 9) Configuration Management Plan (CMP)
- 10) Contingency Plan Test (CPT)
- 11) Memorandum of Understanding (MOU)
- 12) Disaster Recovery Plan (DRP) or Continuity of Operations Plan (COOP)
- 13) Specific, Requested STIG checklists
- 14) ACAS Test Results
- 15) Updated Hardware & Software
- 16) Ports Protocols Services Management (PPSM) Workbook
- 17) System Classification Documentation
- 18) High Level Design\Low Level Design

In establishing a new ATO, the contractor shall initially define the system boundaries based on the client requirements for the Memorandum of Understanding (MOU). The system categorization, such as Moderate, sets the number of controls required based on the NIST Special Publication 800-53 Rev 4.

The contractor shall perform a Data Security Gap Analysis to demonstrate how our system meets the required FISMA Moderate controls. A Plan of Action & Milestones (POA&M) is used, which includes vulnerability scans for remediation.

C.26.1 Reporting/Security Requirement

The Contractor shall submit monthly reports to the Information Security System Officer (ISSO) with key points being addresses, such as, ACAS vulnerability, Microsoft Endpoint Configuration Manager (MECM), and other reports requested by the ISSO that is required for the O&M of the ATO certification.

The Contractor shall perform monthly scans with remediation on a 30/60/90-day cycle based on criticality. In addition, the following documents are completed as part of the standard templates for the achievement of the ATO process:

- 1) Memorandum of Understanding (MOU)
- 2) Disaster Recovery Plan (DRP) or Continuity of Operations Plan (COOP)

The implementation of multi-factor authentication included, user regression testing, rescans shall be required to validate the expected security controls are met, no configuration drift, and unexpectedly modifying the security posture. Those artifacts will be incorporated as part of any updated ATO.

C.26.2 Controls

The Contractor shall have established system controls and security that meet the requirements of the contract to protect government information, in agreement with the ISSO.

Federal mandates applied to this contract include:

- Federal Information Security Management Act (FISMA), the Office of Management and Budget (OMB)
- National Institute of Standards and Technology (NIST) Systems Information Security Standards
- Computer Security Act of 1987; Privacy Act of 1974 (5 USC 552a as Amended)

- Office of Management and Budget (OMB) Circular A-130, Appendix III, “Security of Federal Automated Information Systems”
- Federal Information Security Management Act (FISMA)
- The e-Government Act of 2002
- NIST SP 800-37 Risk Management Framework (RMF)
- NIST SP 800-53 Information Assurance (IA) Controls

Auditing shall be enabled for, user logon, sensitive file/folder access, file/object creation and deletion, security relevant events and system configuration activities and events.

Network appliance and server logs shall be collected and gathered in a Splunk Security Information and Event Management (SIEM) environment for correlation and reporting. All access to Contractor applications is logged into a database and traceable by user access.

The Contractor shall have Core configurations for the environment based on NIST standards, which include:

- All master images are configured with defined security settings such as Remote.
- Procedure Call (RPC) 2 port restriction in the registry and layered with Group Policy Object (GPO) security.
- GPO configuration that prohibit use of Universal Serial Bus (USB) devices.
- Autorun capabilities are disabled on Contractor laptops.
- Microsoft System Center Configuration Manager (SCCM) and deployed to all Contractor servers and workstations to ensure patch and compliance.
- Incident Response Plan (IRP).
- Information System Contingency Plan (ISCP).
- Configuration Management Plan (CMP).
- Security Plan (SP).
- Contingency Plan Test (CPT).
- Vulnerability scans for continuous monitoring with Plan of Action & Milestones (POA&M).

Any vulnerabilities or issues found by the Contractor or government shall be reported upon discover to all the ISSO, Contractor and COR.

The Contractor shall cure any issues found by the ISSO within 10 calendar days and/or provide an update to the ISSO, with milestones on completion of the issue.

C.27 Operation & Maintenance COCO PPN-CP (Contractor-Owned Contractor- Operated/ Private Provider Network/ Client Portal System

The Contractor shall provide a COCO PPN-CP System for this contract. The ISSO will review and approve the COCO PPN-CP System. The COCO PPN-CP System shall have the capability to track referrals and provide reports on status of referrals entered in the system, scheduled appointments with Contractor providers (client name, provider name, appointment date and time), and on what date results were received from the provider.

Appointments shall be scheduled with the Contractor providers within 10 business days of receipt of referrals. The day the referral is fully entered into the COCO-PPN system is considered receipt of referral.

The system should have the capability for batch uploads.

The Contractor shall maintain the multi-factor authentication access to the medical records system.

The Contractor shall perform weekly vulnerability scans. The scanning tools should be configured to detect new vulnerability definitions every 15 minutes.

The Contractor shall monitor all system environment operations status and remediate all issues resulting from the patch installation. This includes: (1) Host-based Intrusion Detection System/Intrusion Protection System (IDS/IPS), (2) anti-virus, and (3) server certificates and patch management.

The Contractor shall have the system operating daily during specified hours. No maintenance may occur during this time unless approved by FOH with an exception for a system breach.

System Hours of Operation shall be as follows:

All times are Pacific Time

Monday – Friday/ 4:00 a.m. – 10:00 p.m.

Saturday / 4:00 a.m. – 6:00p.m.

Sunday / 6:00 p.m. – 12:00 a.m. (midnight)

C.27.1 System Maintenance

The Contractor shall be required to maintain the COCO PPN-CP system and perform weekly and/or monthly maintenance.

Monthly maintenance shall include the following but not limited to:

- 1) Monthly server patching and validations
- 2) Security and compliance remediation
- 3) Ongoing monitoring and review of hardware and network health and performance with adjustments, as needed.
- 4) Ongoing monitoring and review of application health and performance with adjustments, as needed.
- 5) Application fixes and minor upgrades to preserve/enhance operational performance.
- 6) Minor updates to FOH PPN-CP interfaces (portal, billing, invoicing, etc.).
- 7) Management and tracking of systems, fixes, etc.

Maintenance shall be performed outside the System Operation of Hours listed under C.32, unless there is an emergency warranting maintenance at a different time.

The Contractor shall submit to FOH, in writing, a referral to perform maintenance outside the designated maintenance time periods, except in the instance of a system breach or outage during normal operating hours. FOH shall review and approve, the referral, in writing, except in the instance of a system breach or outage during normal operating hours.